

POLICY PAPER

Maternity Without Walls

A Cost-Effective Architecture for Universal Obstetric and Reproductive Health Access in Puerto Rico

Final draft for Ramphis Castro

Infinity Forge | Caguas, Puerto Rico

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Scope note. This paper answers a single question: what is the most cost-effective path to maximum access to obstetric, pregnancy, postpartum, and reproductive health services in Puerto Rico, assuming any regulatory, financing, workforce, or technological option is on the table. It is built from a three-track research effort (Puerto Rico baseline; global evidence on delivery and payment models; frontier technology and workforce pathways) completed on 13 September 2026, with every load-bearing figure sourced in Appendix B. Facts and inferences are separated throughout. Where a figure could not be verified it is marked as such rather than omitted.

Status of this version. The decision draft of 13 September 2026 closed with thirteen items to verify before committing money. This version reports the result of that verification pass (Section 10), corrects the body where the pass found the draft wrong, and lists the items that remain open with the office that holds each answer. The recommendation is unchanged.

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1. The Decision in One Page

Puerto Rico's obstetric system was built for the 42,000 births a year it recorded in 2010. It now serves about 18,000, on a trajectory toward 15,000 by the early 2030s, with a total fertility rate between 0.87 and 0.94 depending on the source and year, the lowest of any jurisdiction under the US flag and the lowest in the Caribbean [6][7][126]. Every visible symptom of the crisis, from thirteen labor-and-delivery closures since 2017 to a cesarean rate above 50% to the fact that only 63 of 468 licensed OBGYNs can be confirmed to attend births, is downstream of one structural fact: a hospital-centric, obstetrician-dependent model of care cannot be financed at this volume, at Plan Vital payment rates, with a physician workforce that is leaving [1][4]. Reopening delivery wards and recruiting obstetricians, the instinctive response, is the most expensive path available and the least likely to work, because it attacks the symptom with the scarcest and costliest input.

The cost-effective path is the opposite one. It moves the default site of care out of the hospital and into the home and the community for the roughly 70% of pregnancies that are low-risk, anchors that care in a licensed and adequately paid midwifery and doula workforce, concentrates intrapartum obstetric capacity into a small number of high-volume regional hubs that can afford 24-hour laborist, anesthesia, and neonatal coverage, backfills the geography with midwife-led community maternity units and an obstetric-readiness layer in every emergency department and ambulance, and pays for all of it by taking the single largest avoidable cost in the system, roughly 3,600 excess cesareans a year, out of the payment stream. Every element has precedent. The Birthplace in England cohort, the CMS Strong Start evaluation, the Cochrane reviews on midwife continuity and continuous labor support, Scotland's community maternity units, California's CMQCC collaborative, and Brazil's Parto Adequado program each supply the evidence for one piece [36][41][42][45][53][54][58]. What has never been done is to assemble them into one system on a 3.2 million-person island under US federal law. That is the design problem this paper solves.

The recommendation. Adopt a three-tier architecture: Tier 0, home and community care led by a named midwife with a doula and a connected monitoring kit for every pregnancy; Tier 1, a network of five to seven midwife-led Community Maternity Units and alongside midwifery units for low-risk birth; Tier 2, six to eight Regional Maternity Hubs with 24-hour laborist, anesthesia, and Level II–III neonatal coverage, financed through standing capacity payments rather than per-birth fees. Wrap the system in an obstetric-readiness layer for every emergency department, EMS unit, and both island municipalities. Finance it through mode-neutral payment for birth, physician-level public reporting of cesarean rates under the Robson classification, midwife payment parity, and a per-pregnancy population payment to accountable Maternity Homes. Issue the implementing regulations that Ley 254-2015 still lacks so that nurse-midwives can practice today, legislate a Midwifery Practice and Birth Center Act, join the Nurse Licensure and Interstate Medical Licensure Compacts, and create a no-fault birth-injury compensation fund. Buy the handful of technologies that are cleared, cheap, and evidenced today (AI-guided handheld ultrasound, HPV self-collection, the E-MOTIVE hemorrhage bundle, freeze-dried plasma, solar-plus-satellite clinic kits) and pilot the rest with evaluation attached.

The order-of-magnitude economics are favorable. The incremental steady-state cost of the full architecture is estimated at \$45–70 million a year across all payers before offsets, against a Plan

Vital budget of roughly \$4.7–5.3 billion; direct offsets from cesarean reduction, avoided preterm and NICU costs, and substitution of midwife for obstetrician time are estimated at \$20–35 million a year, so the net cost is on the order of \$25–45 million, or well under 1% of the public health plan. Through September 2027 the federal government pays 76 cents of every Medicaid dollar of that, which makes the next twelve months the cheapest window Puerto Rico will ever have to build the system [19][20][109]. After FY2027 the Medicaid cliff makes lowering the cost per birth not a policy preference but a fiscal necessity, and this architecture is also the only credible way to do that.

What changed between the decision draft and this version. The thirteen verification items listed in the decision draft were worked on 13 and 14 September 2026 and the results are in Section 10. Four findings alter the text materially. First, Plan Vital's APR-DRG payment went live on 1 January 2026, not 1 October 2025, under state plan amendment PR-26-0001, and it uses Solventum's national relative weights rather than Puerto Rico-specific ones, so the mode-neutral instrument in Section 5 is a policy adjustor or blended birth rate on top of those weights, not a re-weighting [105][106]. Second, Puerto Rico has three accredited OBGYN residencies with about fourteen entry positions a year, not one with five, which moves the workforce problem from training capacity to retention [12][135][136]. Third, twenty-five hospital delivery units remain, not twenty-three; thirteen closed and two opened since 2017 [1]. Fourth, the nurse-midwife title created by Ley 254-2015 has never received implementing regulations from the Junta Examinadora, which is why licensed nurse-midwives cannot work as such in hospitals, and which means the fastest workforce action available is administrative rather than legislative [86]. Nothing found in verification weakens the recommendation; the doula benefit that March of Dimes reports as active remains unconfirmed by any ASES document, which strengthens the case for building it deliberately.

The paper proceeds as follows. Section 2 gives the structural diagnosis with the numbers behind it. Section 3 reviews what the evidence actually supports, and what it does not, model by model. Section 4 lays out the recommended architecture tier by tier, including the island municipalities, postpartum care at home, and reproductive health outside pregnancy. Section 5 covers payment and budget. Section 6 covers the workforce and legal reforms without which the architecture cannot stand. Section 7 sorts the technology into buy, pilot, and ignore. Section 8 sequences the work over 48 months. Section 9 is an adversarial review of how the plan fails. Section 10 reports the verification results and the items that remain open.

2. What Is Actually Broken: A Structural Diagnosis

2.1 A birth system built for 42,000 births now serves 18,000

Live births in Puerto Rico fell from 42,153 in 2010 to 19,169 in 2022, 18,667 in 2023, and 18,085 in 2024 according to the Registro Demográfico's preliminary count released in February 2025, a decline of 57% in fourteen years that the registry's own officials described as probably the steepest since records began in 1888 [6][7]. The provisional 2025 count near 18,000 cited in the decision draft could not be matched to a published registry release and is treated here as an estimate. The total fertility rate has fallen to somewhere between 0.87 and 0.94: CEPAL puts the 2024 figure at 0.94, the lowest in Latin America and the Caribbean [126]. Nothing in the demographic outlook reverses this: out-migration is concentrated among adults of reproductive age, and the cohort of women aged 20 to 34 is shrinking. Any system designed today should be sized for 15,000 births, not 18,000, and certainly not for the 36 delivery wards that existed in 2017.

Hospitals have responded rationally to the volume collapse. Thirteen labor-and-delivery units closed between 2017 and 2026 while two opened, leaving 25 for 78 municipalities, a net reduction of 30% [1][118]. The hospital association counted 29 hospitals with maternity wards as recently as January 2024, so the pace has accelerated [120]. The named closures tell the story. HIMA San Pablo Fajardo closed in August 2022 for lack of obstetricians willing to cover 24 hours a day, taking its neonatal unit with it and redirecting Vieques and Culebra patients to Humacao or Carolina [8]. Hospital Damas in Ponce closed in February 2024 under a one-year temporary authorization from the Departamento de Salud after its births fell from 1,520 in 2012 to 452 in 2023; it has not reopened [9]. Pavía Arecibo requested closure of both obstetrics and its NICU in November 2024, citing falling births [10]; the Secretary of Health later said he never authorized it, and the unit is counted among the closures in 2026, which means it closed without the formal process the law requires [1]. San Carlos Borromeo in Moca and Buen Samaritano in Aguadilla closed in 2023 and were themselves under review for skipping due process [11][120]. The other side of the same process is visible in San Juan: as of September 2026 deliveries are the leading reason for admission at the Hospital Universitario de Adultos, because births displaced from closed units are concentrating in the University District [119]. The pattern is uniform: a unit that must staff obstetrics, anesthesia, and neonatal resuscitation around the clock cannot cover its fixed cost on 400 to 500 births a year at Plan Vital rates. A senior obstetrician put the point directly to the press in 2023: no delivery room in Puerto Rico is at capacity [102].

The inference that matters for design is that closures will continue regardless of policy unless the fixed cost of readiness is paid separately from the variable cost of each birth. A 24-hour obstetric hospitalist line alone costs \$1.3–1.5 million a year at 2025 locum rates of roughly \$3,900 per 24-hour shift [85]; add anesthesia and neonatal call and a unit needs on the order of 1,200 to 1,500 births a year to break even on per-case payment. Eighteen thousand births can support at most twelve to fifteen such units at that threshold, and far fewer if payment is not reformed. The system is heading toward six to ten units by attrition. The policy choice is whether that consolidation happens by unplanned collapse, leaving deserts with no readiness layer, or by design, with the savings from consolidation reinvested in community-based care.

2.2 The workforce arithmetic

The headline OBGYN counts are misleading because they measure different things. There are 468 physicians with active OBGYN licenses, of whom roughly 408 appear to work on the island, but the Centro de Periodismo Investigativo's 2026 census of 220 responding practices found only 63 that confirmed they attend pregnancies through delivery, 56 accepting new patients, and 47 accepting Plan Vital; two-thirds of respondents do not see pregnant women at all [1]. Earlier press estimates put the number of actively delivering obstetricians near 80 out of about 300 board-certified specialists [102]. Only 24 of 78 municipalities have an identifiable delivering OBGYN. Training capacity is larger than the decision draft assumed: Puerto Rico has three ACGME-accredited obstetrics and gynecology residencies, at the University of Puerto Rico (a maximum of five residents a year), San Juan City Hospital (four a year), and Hospital Episcopal San Lucas with Ponce Health Sciences University (five a year), or roughly fourteen entry positions a year [12][135][136]. There is no evidence that more than a fraction of graduates stay to practice obstetrics, and no consistent headcount series from the Junta de Licenciamiento or the AAMC could be obtained to quantify attrition since 2010.

Taken at face value, 63 to 80 delivering physicians for 18,000 births implies 225 to 285 births per physician per year, roughly two and a half to three times the mainland norm, and about 380 Plan Vital births per Vital-accepting physician. That load has two consequences. It makes scheduled delivery, and therefore cesarean, a rational time-management strategy for the individual physician, and it makes recruitment nearly impossible because the job on offer is an unsustainable one. Puerto Rico cannot recruit its way out. The island already trains about fourteen obstetricians a year and still has 63 delivering; the constraint is retention, not slots, and even perfect retention of a decade of graduates would not change the underlying economics of 24-hour coverage.

The midwifery workforce that every comparable health system uses to carry low-risk pregnancy barely exists. Puerto Rico's nursing practice law, Ley 254-2015, does recognize the Enfermera Obstétrica-Partera as an advanced-practice role requiring a master's or doctoral degree in obstetrics-midwifery from an ACME-accredited program, with authority to manage uncomplicated pregnancy and birth, order diagnostic tests, and prescribe vitamins, antibiotics, contraceptives, and immunizations under collaborative protocols with physicians [14][138]. But the Junta Examinadora de Enfermería has never issued the implementing regulations for the title, so a nurse who holds it cannot be credentialed by a hospital as a nurse-midwife [86]; the number of practicing nurse-midwives is unknown, the Asociación de Parteras itself reports that it does not have the figure [13]; no midwifery program operates on the island (UPR trained nurse-midwives from the early 1960s but no midwifery track exists in its current School of Nursing catalog); and direct-entry midwives have no licensing pathway at all. Ley 31-2023 merely designates 5 May as Día de la Partería, and a 2010 bill to create a midwifery examining board passed both chambers and never took effect [13][15]. Eleven certified professional midwives practice, all credentialed off-island. In 2021 the association reported 226 planned home births of which 87% completed at home; a second press account gives 196 women in home-based midwifery care and 171 home births, the same 87% [13][137]. A collaborative OBGYN-nurse-midwife model in Ponce shows the integration is possible in principle [86]. The workforce Puerto Rico needs most is the one it has never built, and the first step to building it is a regulation the executive branch can issue tomorrow.

2.3 The cesarean economy

Puerto Rico's cesarean rate rose from 46.3% in 2018 to 50.5% in 2022 and 50.9% in 2023, against roughly 32% on the mainland and 33.8% for Puerto Rican mothers giving birth in the states [4][5]; the Departamento de Salud's own vital statistics give 50.6% for 2023 [127]. The low-risk (nulliparous, term, singleton, vertex) rate was 49.4% in 2023 versus 28.3% for Puerto Rican mothers on the mainland; March of Dimes reports 47.1% for 2024, the worst of 52 jurisdictions [2]. The payer breakdown is revealing: 51.3% for private insurance, 49.9% for Medicaid, and 27.6% for self-pay births, which means women paying out of pocket are delivered by cesarean at roughly the mainland rate while insured women, whatever the insurer, are delivered by cesarean at nearly double it [4]. Hospital-level data from 2014, cited in the Senate in 2025, showed individual institutions at 66% to 70% [31].

The gap between the self-pay rate and the insured rate is the strongest single piece of evidence that the excess is driven by system incentives rather than by maternal risk. The incentives are identifiable. Payment for cesarean exceeds payment for vaginal birth for both the facility and the physician. A scheduled cesarean takes a predictable hour; attending a labor takes an unpredictable day and night, which matters enormously when one physician carries 280 births. Liability exposure is perceived to be lower for intervening than for waiting. Continuous labor support and midwifery, the two interventions with the strongest evidence for lowering cesareans, are nearly absent. And there is no public reporting, so no hospital or physician bears any reputational cost for a 68% rate.

The cost is large. Taking a 30% overall rate as an achievable five-year target, roughly what California's collaborative achieved as a low-risk rate and well above the WHO reference range, the excess is about 20 percentage points on 18,000 births, or 3,600 cesareans a year. At a facility-plus-professional payment differential of \$2,000–4,000 per case, that is \$7–14 million a year in direct excess payments across all payers, before the downstream costs of repeat cesareans, placenta accreta, longer stays, higher NICU admission, and higher severe maternal morbidity, which is 307 per 10,000 deliveries in Puerto Rico against 93 nationally [2][128]. A conservative all-in estimate of the annual cost of the cesarean excess is \$10–20 million. This is the reservoir from which the rest of the architecture is financed. The surrounding outcome picture is consistent with a system that intervenes heavily and follows up poorly: preterm birth was 12.2% in the latest March of Dimes report card (a grade of F and rank 48 of 52), low birthweight 10.7% in 2023, first-trimester prenatal care initiation 83.3% in 2023, and the Medicaid postpartum visit rate 20.7% in federal fiscal year 2020, the lowest of any Medicaid or CHIP program [2][115][125].

2.4 Geography, the islands, and the deserts

March of Dimes classifies 33.3% of Puerto Rico's municipalities as maternity care deserts, home to roughly 111,000 women of reproductive age and about 3,000 births a year; 34.6% of municipalities have no obstetric clinician, 78.2% have no birthing facility, and every rural municipality lacks one [3]. The Centro de Periodismo Investigativo, using the same March of Dimes data, reports the desert share as 20% of municipalities, which reflects a difference in the definition applied; the paper uses the March of Dimes figure and notes the range [1][3]. About 57,000 women live more than 30 minutes from a birthing hospital, though the island-wide average travel time is a manageable 17 minutes, which is the important nuance: Puerto Rico's problem is not distance in the Montana sense but the combination of moderate distances, unreliable roads and power after storms, and a complete

absence of any obstetric capability at the destination for those in the west, the center, and the islands.

Vieques and Culebra are the extreme case and the design test. No births occur on Vieques; every delivery requires a ferry crossing costing about \$30 round trip, usually with an overnight stay, and a Departamento de Salud obstetrician visits twice a week [17]. The new Vieques hospital, an \$85.6 million project of which FEMA funds \$39.6 million, had not opened as of 28 August 2026: the Secretary of Health said the inauguration remained on track for the end of September 2026 pending equipment, inspections, and licensing, the operator procurement had drawn a single proposal still under negotiation, and the helipad received FAA certification in April 2026 [17][121][122]. No obstetric staffing plan for the facility has been published. Culebra's first-trimester prenatal care initiation collapsed from 100% in 2016 to 53% in 2023 [1]. Without 24-hour obstetric, anesthesia, and neonatal staff, which the islands' combined 60 to 80 births a year cannot justify, the new facility will function as an emergency delivery site, not a maternity unit. The islands therefore need a model that no hospital can provide: home-based prenatal care with remote dating and monitoring, a planned relocation window before term, guaranteed lodging near a hub, hemorrhage stabilization capacity for the unplanned birth, and tele-obstetric backup for the nurse or midwife on the ground.

2.5 Money: Plan Vital, the cliff, and what payment currently rewards

Plan Vital covers about 1.3 million people according to Medicaid.gov's January 2026 enrollment figure (ASES's own materials cite 1.6 million, which includes non-Medicaid populations), roughly 45% of the population, and paid for 11,855 births, 65.8% of the total, in 2024 [2][18]. The program's federal financing is a capped allotment rather than the open-ended match the states receive: \$3.475 billion in FY2025, \$3.645 billion in FY2026, and \$3.825 billion in FY2027 at a 76% federal match, under the Consolidated Appropriations Act of 2023 [19][100][109]. On 1 October 2027 the allotment reverts to the Section 1108(g) cap calculated from the FY2019 base and the match falls to 55%, producing an estimated shortfall of \$1.3–3 billion a year unless Congress acts again [20]. The same 2023 law makes \$300 million a year of the allotment conditional on a physician payment floor of 75% of the Medicare rate, implemented through a directed payment, and a further \$75 million conditional on program-integrity requirements including an asset verification system, with annual access reports to Congress [100][109]. As of September 2026 no extension has been enacted. The Territories Health Equity Act of 2025 (H.R. 6494), filed in December 2025 by the territorial delegates, would eliminate the cap and the FMAP limitation; the Governor has asked Congress for a minimum of \$4.4 billion with 5% annual growth and an 83% match; and the Resident Commissioner has warned that reauthorization may slip into 2027 [113][114]. Puerto Rico is excluded from the \$50 billion Rural Health Transformation Program created by the 2025 reconciliation law, whose funding notice states that only the fifty states are eligible [23][112], and it was not among the fifteen jurisdictions selected for the CMS Transforming Maternal Health model [63]. The rest of that law's Medicaid provisions largely pass Puerto Rico by: the work requirements, six-month redeterminations, and cost-sharing apply to expansion populations the island does not have, and the provider-tax and state-directed-payment limits do not apply to the territories at all, which protects the physician-floor directed payment [110][111]. Whether the law's retroactive-coverage limits reach Puerto Rico could not be established and is carried as an open item.

Within that envelope, three payment facts shape obstetric behavior. First, Plan Vital moved inpatient payment to APR-DRGs on 1 January 2026 under state plan amendment PR-26-0001, three months later than the decision draft stated [105][106]. The design matters for what follows: hospital-specific base rates are built from Plan Vital claims for discharges between October 2021 and September 2023, the relative weights are Solventum's national weights rather than Puerto Rico-specific ones, a \$60 million first-year transition pool is paid as a directed payment, and CMS's approval estimates that the methodology reduces inpatient payments from \$576 million to \$516 million [105][107][108]. Hospitals have already complained publicly about the rates and one has sued ASES in federal court over reimbursement disparities [152][153]. The window is therefore not the setting of weights, which are imported, but the rebasing that follows the first-year pool and the policy adjusters that any DRG system allows: a mode-neutral birth payment can be attached at that point without reopening the methodology. Second, Plan Vital pays for live-video telehealth at parity, and Ley 8-2025 now requires insurers and ASES to pay for services delivered electronically, digitally, or by telephone at the in-person rate, which is statutory parity for audio-only care [22][117]; but no ASES normative letter adopting the remote patient monitoring codes could be found, so every home-monitoring model in Section 7 currently has no fee-for-service path and must run through managed-care capitation or an ASES policy change. Third, the actual global obstetric fee paid to physicians by the Vital managed-care organizations is not published anywhere, and an exhaustive search of ASES documents, professional-society statements, and the press found no dollar figure; this remains the single most important number missing from the public record, and the first thing ASES should release. What is documented is the payment environment around it: one Vital MCO carried \$170.7 million in unpaid claims in 2025, and half of a sample of specialists refused to accept it [25].

March of Dimes records that Puerto Rico's Medicaid agency is actively reimbursing doula care, on the authority of the National Health Law Program's doula tracker, and that it requires and pays for postpartum depression screening at well-child visits [2]. No ASES normative letter, rate, enrollment mechanism, or utilization figure for a doula benefit could be found, no managed-care organization lists it as a covered service, and doulas are neither licensed nor certified in Puerto Rico; the paper therefore treats the benefit as unverified rather than as an asset. The fact that a bill filed on 11 September 2026 seeks to force hospitals to admit doulas to delivery rooms, because hospitals read the 2006 companion law (Ley 156-2006) as permitting only one person, confirms that whatever exists on paper is inaccessible in practice [16][116]. The twelve-month postpartum coverage extension adopted by every state except Arkansas does not appear in KFF's tracker for Puerto Rico, which lists only the US Virgin Islands among territories, and none of the Puerto Rico state plan amendments approved since 2024 addresses it [24]. Until ASES confirms otherwise, Plan Vital should be assumed to drop postpartum women at 60 days.

2.6 The legal climate

Three features of the legal environment bear directly on obstetric supply. The first is liability. Ley 104-1955 caps claims against the Commonwealth at \$75,000 per person and \$150,000 per incident; Ley 229-2004 extended that protection to physicians in obstetrics, orthopedics, general surgery, and trauma, but only in public institutions; and Ley 94-2023 extended it to residents and trainees in ACGME-affiliated programs, explicitly citing physician emigration [28][140]. Private obstetricians serving Plan Vital patients enjoy no such protection, so the highest-risk specialty carries uncapped exposure while treating the lowest-paying patients. The statutory primary policy that SIMED writes

carries limits of \$100,000 per incident and \$300,000 aggregate; obstetric premium and claim-frequency data could not be obtained from SIMED or the insurance commissioner and remain a verification item [139].

The second is the 2025–2026 wave of reproductive legislation. Ley 122-2025, signed 30 October 2025, imposes written parental consent, a guardian's presence, and mandatory Departamento de la Familia referral for abortion in girls aged 15 and under; Ley 183-2025, signed 20 December 2025, inserts juridical personhood from conception into the Civil Code; and Ley 18-2026, signed 12 February 2026, defines the fetus at any stage of gestation as a human being in the Penal Code [29][91][131][132]. More than thirty organizations have warned of legal uncertainty for clinicians managing miscarriage, ectopic pregnancy, and fetal anomaly [30]. No court challenge, injunction, or implementing regulation for any of the three laws was found as of September 2026. Abortion itself remains legal under *Pueblo v. Duarte Mendoza* and the Commonwealth constitution's privacy right, with no statutory gestational limit [133]; four clinics operate under Departamento de Salud regulation (IELLA, Darlington, Women's Medical Pavilion, and the Centro de Planificación Familiar) [130], and the last published counts show 4,201 procedures in 2018, 3,596 in 2019, and 3,751 in 2020, from clinics only [129]. No quantified effect on OBGYN retention exists, but a personhood statute layered on uncapped liability is, as an inference, a plausible additional push factor for a specialty already leaving; at minimum it argues for explicit clinical safe-harbor language, which this paper recommends in Section 6.

The third is the legislative record on transparency. In 2025 and 2026 the Senate Health Commission issued negative reports on PS 225, which would publish cesarean rates by physician and which the Independence Party has filed in some form since 2008, and PS 445, which would create an obstetric violence cause of action, was referred to the Judiciary Commission and not approved; the Senate resolution to investigate delivery-ward closures was archived [1][149][150]. PC 669, which would require quarterly hospital reporting of births by mode and physician on a public Departamento de Salud portal, with the Robson classification added at the recommendation of the Instituto de Estadísticas, has not advanced [32]. PS 435, which would guarantee a woman direct access to an OBGYN through twelve months postpartum, is pending and would be moot under the architecture proposed here, which gives her a midwife instead [151]. The political economy is clear: the hospital association and the medical college have so far blocked the transparency measures that the evidence in Section 3 identifies as among the cheapest and most effective interventions available. Any strategy must either win that fight in the Legislature or route around it through ASES contracting authority, which Section 5 proposes.

2.7 Surveillance is degrading at the wrong moment

Puerto Rico's maternal mortality ratio is reported by March of Dimes at 59.2 per 100,000 births for the pooled 2019–2023 period, two and a half times the national figure; the method (NCHS mortality data with Puerto Rico's contribution supplied by the Departamento de Salud, pooled over five years) is documented, though the figure itself could not be re-read from the report card during verification [2]. The Departamento de Salud's surveillance system identified 8 deaths related to pregnancy, childbirth, and the postpartum period in 2023, and preliminary figures released in March 2025 pointed to 22 in 2024 [1]. On 18,000 births those counts translate to roughly 43 and 121 per 100,000, and the swing illustrates how unstable single-year ratios are at this volume; the 2024 figure

is preliminary and may include pregnancy-associated deaths that a review committee would exclude. Puerto Rico has had a statutory surveillance system since 2005 and a Maternal Mortality Review Committee under Ley 186-2016 [99], but no public review report with cause-of-death and preventability findings exists for any year, and Puerto Rico does not appear among the named recipients of the federal ERASE MM program that funds such committees. That program lost its appropriation on 30 September 2025 [33], and CDC dismantled the PRAMS surveillance team in 2025 with grants expiring in April 2026 [34]. Severe maternal morbidity stands at 307 per 10,000 deliveries, more than three times the national rate [2][128]. Congenital syphilis rose from 5 to 10 cases a year through 2022 to 26 in 2023, 29 in 2024, and 33 in 2025, a record; Orden Administrativa 585 of 18 January 2024 already mandates syphilis screening each trimester and at delivery [35] [134]. The island is losing its instruments precisely as the readings turn bad. Any architecture must therefore include a locally financed data spine; Section 7.4 specifies it.

Table 1. The ten numbers that define the problem

Indicator	Value	Year	Source
Live births	18,085 (down from 42,153 in 2010)	2024	[6][7]
Total fertility rate	0.94 (CEPAL); 0.87 in the decision draft, unverified	2024	[7][126]
OBGYNs confirmed attending deliveries / accepting Plan Vital	63 / 47 (of 468 licensed)	2026	[1]
Hospital labor-and-delivery units	25 (down from 36 in 2017; 13 closed, 2 opened)	2026	[1]
Cesarean rate, overall / low-risk	50.9% / 49.4%	2023	[4]
Births paid by Plan Vital	65.8% (11,855)	2024	[2]
Maternal mortality (pooled) / severe maternal morbidity	59.2 per 100k / 307 per 10k	2019–23 / 2022	[2][128]
Municipalities that are maternity care deserts	33.3%; 78.2% lack a birthing facility	2024	[3]
Federal Medicaid allotment and match	\$3.825B at 76% through 30 Sep 2027; then FY2019-based cap and 55%	FY2027	[19][20] [109]
OBGYN residency entry positions per year	About 14 across three ACGME programs	2026	[12][135] [136]

3. What the Evidence Says Works, and What It Does Not

This section is deliberately adversarial. Maternal health policy is crowded with interventions that are popular, humane, and unproven, and a plan that stakes public money on them will fail in the evaluation. The standard applied here is: what is the effect size, how strong is the evidence, what does it cost, and what preconditions did the evidence depend on that Puerto Rico currently lacks.

3.1 Midwifery-led continuity and midwifery units

The 2024 Cochrane review of midwife continuity of care, pooling 17 randomized trials and 18,533 women, finds a relative risk of 0.91 for cesarean birth, 1.05 for spontaneous vaginal birth, 0.89 for instrumental birth, and 0.83 for episiotomy, all at moderate certainty, with no significant effect on preterm birth, fetal loss, or neonatal death [36]. The honest reading is that midwife continuity is safe, modestly reduces intervention, and improves experience; the earlier claims of a 24% preterm reduction did not survive the update. The trials were run in the United Kingdom, Australia, and Ireland, in systems where midwives already had legal autonomy, hospital privileges, and integrated transfer. The New Zealand cohort of 244,000 births, where 91.5% of care was midwife-led, found higher adjusted odds of low Apgar and neonatal encephalopathy in the midwife-led arm, with no difference in perinatal mortality; the study could not separate midwife-led women who received obstetric input [37]. It is confounded but it is the strongest caution in the literature against near-universal autonomous midwifery for first births without strong obstetric integration.

On cost, MACPAC cites midwife-attended low-risk births averaging \$2,421 less than obstetrician-led care [39]. On feasibility, midwives attend about 12% of US births with roughly 14,000 certified nurse-midwives nationally, but Alaska and New Mexico reach about 30% of Medicaid births through licensure, payment parity, and rural necessity [39]; roughly half of state Medicaid programs pay nurse-midwives 100% of the physician fee and 18 states plus DC also pay non-nurse midwives [40]. The transferable target for Puerto Rico is the New Mexico level, 30–40% of births midwife-attended within a decade, not the New Zealand level. The preconditions are independent practice authority, hospital privileges with written transfer protocols, payment parity, and a local education pipeline; Puerto Rico has none of the four at scale. The characteristic failure mode is a midwife who is licensed but cannot get privileges or a managed-care contract, which the Minnesota birth-center record documents at length.

3.2 Birth centers and home birth: the integration condition

Two large studies define what is known. Birthplace in England followed about 64,000 low-risk births across settings and found an adverse perinatal outcome rate of 4.3 per 1,000 overall, no significant excess for freestanding or alongside midwifery units for either first or subsequent births, and a significant excess for planned home birth of first babies (9.3 versus 5.3 per 1,000 in obstetric units), with no excess for subsequent babies; cost per birth was £1,142 in an obstetric unit versus £780 at home with midwifery units in between [41]. The CMS Strong Start evaluation of roughly 46,000 Medicaid women found that birth-center participants, compared with propensity-matched Medicaid comparators, had preterm birth of 6.3% versus 8.5%, cesarean of 17.5% versus 29%, low birthweight of 5.9% versus 7.4%, and Medicaid spending \$1,759 (21%) lower for the birth and

\$2,010 (15%) lower through the first year of life [42]. The group prenatal care and maternity-care-home arms of the same evaluation showed no significant improvement.

Home birth is where the evidence forks on a single variable: integration. Hutton's 2019 meta-analysis of 500,000 intended home births found no mortality difference in well-integrated systems (odds ratios near 1.0 for both first and subsequent births) but a point estimate three times higher for first births in less-integrated systems, and the Oregon data that underlie US concern showed perinatal mortality of 3.9 versus 1.8 per 1,000 for planned out-of-hospital birth [43][44]. The National Academies' 2020 committee distilled five integration conditions: a regulated system, covered options, providers qualified to manage first-line complications, seamless transfer, and appropriate risk assessment [43]. Puerto Rico's current home-birth sector, a small group of off-island-credentialed midwives operating without a licensing law, hospital agreements, or coverage, is the unintegrated case. The policy conclusion is not to discourage home and birth-center birth, which the evidence shows is safe and \$2,000 cheaper when integrated, but to build the integration before scaling it, and to steer first births toward alongside units and hubs rather than home.

3.3 Labor support, doulas, and community health workers

The Cochrane review of continuous labor support (24 trials, 15,347 women) finds a relative risk of 0.75 for cesarean, 1.08 for spontaneous vaginal birth, 0.62 for low five-minute Apgar, and 0.69 for a negative birth experience, with the effect largest when the supporter is a doula rather than hospital staff, though the evidence is graded low quality and many trials came from settings with no companion at all [45]. Twenty-six states and DC now reimburse doulas under Medicaid at maxima ranging from under \$800 to over \$3,000 [47]. The failure mode is documented in MACPAC's case studies: Minnesota and New Jersey achieved use in a tiny fraction of births despite demand, because of low rates, physician-referral requirements, administrative burden, and workforce scarcity [46]. Kozhimannil's modeling suggests Medicaid doula coverage is cost-saving through averted cesareans and preterm births, but only at meaningful uptake. Given that Puerto Rico's hospitals currently refuse to admit doulas as a second companion, the design must pair the benefit with a legal right of access, a rate of at least \$1,500 with a labor-attendance component, direct enrollment, and a locally trained workforce.

Home visiting by nurses has a different profile. The Washington State Institute for Public Policy estimates Nurse-Family Partnership returns \$1.48 per dollar, \$21,379 in benefits against \$14,459 in cost per participant, with a 65% probability of positive net value [92]; it is expensive per family and most benefits accrue outside the health budget over decades. It belongs in the architecture as a targeted service for first-time mothers with high social risk, not as a universal layer. Group prenatal care (CenteringPregnancy) showed a 36% lower preterm risk and net savings in a South Carolina Medicaid propensity study [93], but the largest quasi-experimental test, Strong Start, found no outcome benefit [42]; it is cheap, popular, and plausibly cost-neutral, and it should be offered without banking preterm savings on it.

3.4 Prenatal redesign, telehealth, and remote monitoring

The American College of Obstetricians and Gynecologists' April 2025 clinical consensus formally abandons the fixed schedule of 12 to 14 in-person prenatal visits in favor of risk-tailored schedules with telehealth and home monitoring [48]. That guidance removes the professional-standards

objection to the model this paper proposes. The evidence on what it does is more nuanced than advocates suggest. The Mayo Clinic OB Nest randomized trial cut visits from 13.7 to 9.9 with higher satisfaction and maintained quality, but personnel cost rose 34% because nursing time increased by 237 minutes per patient while physician time fell by only 54 [49]. Reduced-visit hybrid care is therefore an access and workforce-substitution tool, not a cost saver, unless payment is decoupled from visits; in Puerto Rico, where the scarce input is the physician hour, substitution is exactly the point.

Postpartum remote blood pressure monitoring is the intervention most often oversold. The Penn Heart Safe Motherhood program's implementation data are striking: blood pressure capture rose from 44% to 92%, readmissions fell from 5% to 1%, and the racial gap in follow-up closed [50]. But the 2025 meta-analysis of the four randomized trials, 714 women, found no statistically significant reduction in hypertensive readmission (5.3% versus 11.8%) and a doubling of emergency department visits (9.0% versus 4.4%) [51]. The correct conclusion for Puerto Rico is that home blood pressure monitoring is a cheap and effective way to get blood pressure measured in the ten days after birth, which matters when hypertensive disorders affect 11% of pregnancies and have risen 21% [1], but it must be paired with a nurse triage hub that can manage a reading by phone, or it will send women to emergency departments that have no obstetric capability. Continuous glucose monitoring for gestational diabetes, across eleven trials, changed birth weight slightly and nothing else [87]; it should not command a premium over glucometers.

3.5 Cesarean reduction at system level

The Cochrane review of non-clinical interventions gives high-certainty evidence for two levers: guidelines with mandatory second opinion reduce cesareans by 1.9 percentage points and guidelines with audit and feedback by 1.8 points, while opinion-leader education reduced elective cesareans from 66.8% to 53.7% in one high-cesarean setting [52]. Those are single-digit effects and Puerto Rico needs twenty points, so the question is what combinations have achieved more. California's CMQCC collaborative took the statewide low-risk rate from 26.0% to 22.8% between 2016 and 2019 while severe newborn complications fell, using physician-level data feedback, expanded labor support, public reporting including on consumer platforms, and payer pressure [53]. Brazil's Parto Adequado program, the closest analog to Puerto Rico's private-sector baseline, raised the vaginal birth rate in 28 high-cesarean private hospitals from 21.5% to 34.8% in two years, a 13-point absolute shift, with gains tightly correlated to the depth of hospital governance change [54].

Payment mechanisms alone have a weaker record. Minnesota's blended payment produced a three-point decline in one analysis and none in the state's own data, and was rescinded; Texas nonpayment for early elective delivery cut such deliveries 10–14%; Washington's pay-for-performance took early elective delivery from 15.5% to 1% [55]. The pattern across the literature is that measurement and exposure change physician behavior more reliably than marginal price changes, and that the two together, mode-neutral payment removing the incentive plus public reporting adding a reputational cost, are the evidence-backed combination. Observational data associate laborist coverage and family-physician presence on labor units with lower cesarean rates [59]; both are staffing choices Puerto Rico's hubs can make. For costing, each avoided cesarean saves roughly \$2,000–4,000 in first-year payments, and a 20-point reduction is worth \$10–20 million a year.

3.6 Regionalization and remote obstetrics

Two bodies of evidence pull in opposite directions and both are right. Kozhimannil and colleagues found that after rural obstetric closures, births in hospitals without obstetric units rose from 0.4% to 2.5% in remote counties, with more out-of-hospital and preterm births [57]. Fischer, Royer, and White's event study found that closures increased travel distance but produced no detectable increase in infant mortality, a slight decline in cesareans, and no harm at longer distances [56]. The reconciliation is that consolidation into well-run regional units is not inherently harmful, and may improve quality, provided the unplanned birth in the wrong place is prepared for; the harm concentrates in unready emergency departments and ambulances. Low-volume units are not a safe haven: hospitals with 10 to 110 births a year had higher odds of low-risk and non-indicated cesarean than higher-volume units [101]. Consolidation to a smaller number of high-volume hubs is therefore defensible on safety grounds if, and only if, the readiness layer is funded.

What fills the gap between hub and home is the midwife-led community unit. Scotland's community maternity units, located about 40 miles from an obstetric unit with no obstetrician on site, delivered 78.6% of eligible women locally with a 16.9% intrapartum transfer rate, rare severe morbidity, and a 0.3% third-degree tear rate [58]. Norway's fødestue units follow the same template. Family physicians with obstetric training are the other rural workhorse: only 2.1% of US family physicians perform cesareans, but they are four times more likely to practice in small rural counties and twice as likely in counties with no OBGYN [59]. Obstetric readiness for hospitals without delivery units has a standard-of-care toolkit from the Alliance for Innovation on Maternal Health but essentially no outcome evidence [60]; Arkansas's ANGELS Medicaid telehealth hub documented shifting very-low-birthweight deliveries to NICU hospitals without publishing cost-effectiveness [94]; HRSA's RMOMS rural networks improved first-trimester and postpartum visit rates without reporting cesarean, NICU, or cost results [95]. Regionalization plus readiness is standard-of-care logic supported by mechanism and observational data, not trial evidence, and should be presented as such.

3.7 Payment reform: what has and has not moved outcomes

The record on maternity episode bundles is poor and should discipline the design. Arkansas's perinatal episode excluded about half of Medicaid births, left cost per episode flat over seven years (\$3,568 to \$3,616), produced no differential cesarean change, saw the share of providers meeting the commendable threshold fall from 27% to 10%, and was phased out in 2021 [61]. Tennessee reported 9.2% savings per episode but never formally evaluated outcomes and waived risk-sharing in 2020 [62]. The Netherlands' integrated maternity bundles across six regions produced no change in maternal or child health after five years and only slightly reduced spending [38]. The CMS Transforming Maternal Health model is the most thoughtful current design, with pillars for midwives, birth centers, and doulas, low-risk cesarean reduction, and whole-person care, but its payment mechanics are still being built and Puerto Rico is not in it [63]. Pennsylvania's rural global budgets stabilized finances for some hospitals with mixed overall results and no evidence yet on preserving obstetric lines specifically [64].

The inference this paper draws is specific. Bundles attributed to the delivering physician cannot move a cesarean rate driven by prenatal scheduling culture and cannot pay for the fixed cost of readiness. Three payment instruments have better logic and better evidence: mode-neutral facility and professional payment for birth, which removes the marginal incentive; standing capacity

payments for units that maintain 24-hour readiness, which solves the fixed-cost problem that per-case payment cannot; and a population-based per-pregnancy payment to an accountable entity that spans prenatal through twelve months postpartum and is indifferent to site of care, which funds midwives, doulas, and home monitoring out of the money that would otherwise buy physician visits and cesareans. Section 5 builds on these three.

3.8 Reproductive health beyond pregnancy

Outside pregnancy the evidence is stronger and the interventions cheaper. Thirty states and DC allow pharmacists to prescribe contraception [66]; Puerto Rico does not. Telehealth medication abortion, in a study of more than 6,000 patients, needed no follow-up care in 98% of cases with a 0.2% serious adverse event rate, equivalent to in-person care [67]; a quarter of US abortions are now provided this way, and Puerto Rico's constraint is provider supply, not law. HPV self-collection in clinical settings was FDA-approved in May 2024 and at-home self-collection in May 2025, and federal guidance now recommends offering it [68][69]; it requires no OBGYN and no speculum, bills under existing laboratory codes, and is the single highest-yield preventive intervention available to a system with 47 Vital-accepting gynecologists. Immediate postpartum long-acting contraception, when unbundled from the delivery payment as South Carolina did in 2012 and most states have since, raises uptake regardless of insurance and improves birth spacing. Puerto Rico's own Zika-era Z-CAN program expanded contraception access points from 13 to 139 sites and served 29,221 women in eighteen months, demonstrating that rapid island-wide reproductive health deployment is operationally possible here [90]. Fertility care remains uncovered, with about five reproductive endocrinologists and cycle costs of \$10,000–18,000; a Senate bill to mandate coverage passed 18 to 5 in 2024 and died in the House [96].

3.9 Evidence scorecard

Table 2 ranks the models by strength of evidence, cost-effectiveness, the access gain they would deliver in Puerto Rico specifically, and the weight of preconditions they require. Evidence grades are A for randomized trials or Cochrane syntheses, B for strong quasi-experimental designs, C for observational or pre-post studies, and D for descriptive evidence.

Table 2. Delivery and payment models ranked for Puerto Rico

Model	Evidence	Cost-effectiveness	Access gain in PR	Preconditions
Midwife continuity of care	A (cesarean RR 0.91, moderate certainty)	High: \$1,700–2,400 saved per birth	High: the only scalable workforce	Heavy: licensure, privileges, parity, pipeline
Alongside and freestanding midwifery units	B (Birthplace; Strong Start)	High: –\$2,010 per Medicaid birth	Medium-high; multiparas first	Heavy: licensure, accreditation, transfer agreements
Continuous labor support and doulas	A (cesarean RR 0.75; low quality)	High if uptake reached	Medium: uptake historically under 5%	Moderate: rate at or above \$1,500, right of access

Model	Evidence	Cost-effectiveness	Access gain in PR	Preconditions
Audit, feedback, second opinion, public reporting	A (–1.8 to –1.9 pp) plus B (California –3.2 pp; Brazil +13 pp vaginal)	Very high: near-zero marginal cost	Quality gain, not access	Light-moderate: data spine, payer leverage
Mode-neutral (blended) birth payment	C (Minnesota mixed)	Unknown alone; strong in combination	Indirect	Moderate: ASES contracting, APR-DRG weights
Tailored prenatal schedule with telehealth	A (OB Nest; small)	Neutral to negative on cost	High: substitutes for physician time	Light: decouple payment from visits
Postpartum home BP monitoring	A (4 RCTs; null on readmission)	High on process; unproven on outcomes	High: all hypertensive births	Light: RPM code, nurse triage hub
Regionalization plus obstetric readiness	C/D	Unknown; mechanism-based	High: 54 municipalities without OB	Moderate: transport, training, tele-OB
CNM-led community maternity units	C (Scotland cohort)	Likely favorable	High for west, center, islands	Heavy: workforce, backup, licensure
Maternity episode bundles (physician-attributed)	B (Arkansas null; Tennessee unevaluated)	Low demonstrated	None	Heavy
Capacity payments and rural global budgets	C (Pennsylvania mixed)	Unknown	Indirect: keeps hubs open	Heavy
Pharmacist, OTC, telehealth contraception; HPV self-collection	A/B	High	High	Light: scope-of-practice rules
Immediate postpartum LARC unbundling	B	High	Medium	Light: DRG carve-out
Nurse home visiting (NFP)	A	Medium: \$1.48 per \$1	Low: targeted	Moderate
Group prenatal care	A/B mixed	Uncertain	Medium	Light

4. The Recommended Architecture

The architecture has three tiers and a readiness layer that wraps all of them. It is designed for 15,000 births a year, for a physician workforce that will not grow, for a payer that will have less money after 2027, and for an island whose grid and roads fail every few years. It borrows the midwife-anchored community model from the United Kingdom, Scotland, New Zealand, and Sri Lanka, the birth-center integration standard from the National Academies, the hub-and-spoke regionalization logic from Levels of Maternal Care, and the transparency-plus-payment approach from California and Brazil. It does not borrow the New Zealand assumption that obstetric input is optional for first births, nor the mainland assumption that every hospital should deliver babies.

4.1 Design principles

Seven principles govern every subsequent choice. First, size for 15,000 births: capacity that assumes a rebound in fertility is capacity that will be paid for and closed. Second, pay for readiness separately from births: the fixed cost of 24-hour obstetric, anesthesia, and neonatal coverage is a public good and cannot be recovered from per-case payment at Puerto Rico's volume. Third, the default clinician for a low-risk pregnancy is a midwife and the default site of prenatal and postpartum care is the home or the community clinic; the hospital is where risk goes, not where pregnancy goes. Fourth, integration before scale: no expansion of out-of-hospital birth until licensure, transfer agreements, and coverage exist, because the evidence is unambiguous that unintegrated home birth harms first babies. Fifth, measure publicly: every hospital and every physician's cesarean rate, by Robson group, on a public dashboard, because measurement has moved behavior where price changes have not. Sixth, buy only what is cleared, coded, and evidenced; pilot the rest with an evaluation design attached before the first dollar is spent. Seventh, resilience is table stakes: every site that hosts maternity care has power and connectivity that survive a hurricane, or it is not a site.

4.2 Tier 0: Home and community care for every pregnancy

Tier 0 is where roughly 70% of pregnancies should receive essentially all of their prenatal and postpartum care and where every pregnancy begins. Enrollment happens at the moment of pregnancy confirmation, at a pharmacy, a federally qualified health center, a Centro de Diagnóstico y Tratamiento, or through a Plan Vital hotline, and it triggers three assignments within a week: a named midwife (or, during the transition years when midwives are scarce, a prenatal nurse working under a midwife-led protocol), a doula by 28 weeks, and a connected home monitoring kit. The kit is deliberately simple and cheap, on the order of \$150 per pregnancy: a validated upper-arm blood pressure cuff, a scale, urine dipsticks, a thermometer, a pulse oximeter, and enrollment in a Spanish-language nurse hub reachable by WhatsApp, text, and voice 24 hours a day. The hub, staffed by nurses and midwives at the regional hubs described below, is the clinical brain of Tier 0; it reads every home measurement, triages every symptom, and decides who needs to be seen, where, and how fast.

The visit schedule follows the 2025 ACOG consensus rather than the legacy fourteen-visit model: eight to nine contacts for a low-risk pregnancy, with roughly half by video or phone and the in-person contacts delivered at the nearest FQHC or CDT by the midwife [48]. Puerto Rico's FQHC network, roughly twenty organizations operating on the order of a hundred sites with a presence in nearly

every municipality, is the physical backbone; it already holds HRSA funding, sliding-fee obligations, and community trust, and Direct Relief has already fitted fifteen of its clinics with solar and storage [81]. Each FQHC and CDT site hosting prenatal care is equipped with an AI-guided handheld ultrasound probe. The Butterfly Gestational Age Tool, cleared by FDA on 27 March 2026 under 510(k) K252148, allows a nurse or midwife with one day of training to date a pregnancy between 16 and 37 weeks with accuracy equal to a credentialed sonographer at a hardware cost of about \$4,000 plus \$420 a year [70][71][72]. It does not replace the 18–22 week anatomy survey, which is delivered at the hub or by a mobile sonography van on a fixed municipal circuit. Routine laboratory draws happen at the FQHC; syphilis screening each trimester and at delivery, already mandated by administrative order [35], becomes a tracked Tier 0 metric.

Risk stratification is continuous rather than a one-time triage. The hub reviews every enrollee's record at booking, at 20 weeks, at 28 weeks, at 36 weeks, and whenever a home measurement or symptom crosses a threshold, and moves the pregnancy up a tier when it does. Women with hypertension, diabetes, prior cesarean, multiple gestation, or other risk factors, roughly 30% of the total, receive obstetric or maternal-fetal medicine co-management by video from the hub with in-person visits at the hub as needed, while keeping their midwife and doula. Group prenatal care in the CenteringPregnancy format is offered at every FQHC for those who want it, without any assumption that it will lower preterm birth. First-time mothers with high social risk are referred to a nurse home-visiting program modeled on Nurse-Family Partnership, sized to perhaps 1,000 families a year [92].

The postpartum half of Tier 0 is, in cost-effectiveness terms, the most valuable and most neglected. Every woman receives a home visit by her midwife or a maternity nurse within 48 hours of discharge and at least two more in the first ten days, adapted from the Dutch kraamzorg model, in which a trained maternity care assistant provides a statutory minimum of 24 hours of home care over the first eight to ten days after birth under the supervision of the community midwife, covered by basic health insurance [103][104]. Women with hypertensive disorders keep the blood pressure cuff and transmit readings twice daily for ten days to the hub, which manages medication by phone under protocol; the evidence shows this raises measurement from 44% to 92% of women and, when paired with a nurse who can act on the reading, avoids readmissions, and the hub is what prevents the doubling of emergency visits seen in trials without one [50][51]. Postpartum depression screening, already required and paid under Plan Vital [2], happens at the home visit and at well-child visits, with a warm hand-off to behavioral health by video. Contraception counseling begins prenatally and immediate postpartum long-acting contraception is available at every hub and community unit. Lactation support is delivered in the home and by video. Coverage continues for twelve months.

4.3 Tier 1: Community Maternity Units and alongside midwifery units

Tier 1 is where low-risk labor and birth happen for women who choose it and qualify for it, in two forms. Alongside midwifery units are midwife-led birth suites located inside or adjacent to each regional hub, sharing its walls and its obstetric backup, and they are the recommended setting for low-risk first births, where Birthplace found no excess risk in alongside units but did find one at home [41]. Community Maternity Units are freestanding, midwife-led birth centers in the deserts, modeled on Scotland's units and Norway's fødestuer: no obstetrician on site, a written transfer protocol with a hub within 30 to 45 minutes by road, an ambulance agreement, tele-obstetric consultation on

demand, and eligibility criteria that steer first births toward the alongside units and reserve the freestanding units mainly for women who have given birth before [58]. Both forms offer water immersion, freedom of movement, intermittent auscultation, a doula and a companion, delayed cord clamping, skin-to-skin, and rooming-in as the default rather than the exception. Both are licensed under the Birth Center chapter of the legislation proposed in Section 6, accredited by the Commission for the Accreditation of Birth Centers, and paid a facility rate under Plan Vital as federal law already requires for licensed birth centers.

Five to seven Community Maternity Units are enough. Candidate locations follow the deserts and the closures: the northwest around Aguadilla and Moca, where two units have closed; the west-central mountains around Utuado, Jayuya, or Adjuntas; the southeast around Guayama; the east around Fajardo, where the Caribbean Medical Center still delivers with two obstetricians and could evolve into a hybrid; and, once volume justifies it, the north-central corridor if Arecibo's status is not resolved. Each unit needs roughly six to eight midwives, two to three maternity nurses, and a nurse-manager, and can handle 150 to 400 births a year. Their economics are favorable precisely because they do not carry the 24-hour physician and anesthesia standby that sinks a small hospital unit; the Strong Start evaluation's \$2,010 saving per Medicaid birth is the relevant benchmark [42]. They are also, not incidentally, the only realistic way to bring birth back to the west and center of the island, because no hospital will reopen a delivery ward there at current volumes.

4.4 Tier 2: Regional Maternity Hubs

Tier 2 concentrates intrapartum obstetric capability into six to eight Regional Maternity Hubs, each handling 1,500 to 3,000 births a year, which is the volume at which 24-hour laborist, in-house anesthesia, and Level II or III neonatal coverage can be staffed and justified. Geographically the hubs map onto the metropolitan area (two or three, including the University District Hospital complex as the Level IV referral center), Bayamón, Caguas, Ponce, Mayagüez, the Arecibo–Manatí corridor, and Humacao. Designation should be competitive and conditional, run by ASES and the Departamento de Salud jointly, against published criteria: planned volume, 24-hour obstetric hospitalist coverage, in-house anesthesia, neonatal level, an alongside midwifery unit on site, midwives with full privileges, vaginal birth after cesarean offered, doula access guaranteed, the Alliance for Innovation on Maternal Health hemorrhage and hypertension bundles and the E-MOTIVE protocol implemented, monthly Robson-classified reporting, Baby-Friendly designation or a timeline to it, and a duty to staff the island-wide tele-obstetric hub on rotation.

The hubs are the only tier where the paper recommends spending money on obstetricians, and it does so through a salaried laborist model funded by the standing capacity payment described in Section 5, not through per-delivery fees. A laborist model changes the job on offer from 280 births of unpredictable private call to a shift-based hospital position with a predictable income, which is the only version of the job Puerto Rico has any realistic chance of recruiting into, and observational evidence associates laborist coverage with lower induction and cesarean rates. The hubs also house the maternal-fetal medicine capacity, the anatomy-scan sonography service, the tele-obstetric and tele-neonatal consult line, the nurse hub for Tier 0, the training simulation program, and the maternity lodging described below. Consolidation to this footprint is defensible on the evidence, which finds no infant mortality penalty from closures where distance is moderate, provided the

readiness layer is funded [56]; without it, consolidation is the unplanned collapse the island is already experiencing.

4.5 The readiness layer: every emergency department, every ambulance, every island

Between 3% and 5% of births in a consolidated system will still arrive at the wrong door: an emergency department without obstetrics, an ambulance, a CDT, a home without a planned attendant. The readiness layer exists for them, and it is cheap relative to what it prevents. Every hospital emergency department and every CDT receives the AIM Obstetric Emergency Readiness kit, quarterly simulation training in imminent birth, shoulder dystocia, hemorrhage, eclampsia, and neonatal resuscitation, a stocked obstetric emergency cart, and a direct line to the tele-obstetric hub [60]. Every EMS unit receives obstetric and neonatal training and a birth kit. Every CDT and both island facilities stock tranexamic acid, uterotonics, calibrated blood-collection drapes, and, newly possible since FDA licensure in July 2026, freeze-dried plasma that stores at room temperature [77] [78]; the E-MOTIVE bundle, proven in a 210,000-woman cluster trial and cost-effective, becomes a Departamento de Salud administrative order for every site that could conceivably manage a birth.

The tele-obstetric hub is a single island-wide service, staffed 24 hours a day by an obstetric hospitalist and a neonatologist on video, rotated among the Regional Maternity Hubs and funded through their capacity payments. It supports the Community Maternity Units, the emergency departments, EMS, the islands, and the Tier 0 nurse hub. The evidence for provider-to-provider obstetric telehealth is thin on outcomes but the mechanism is sound and the cost is low [75][76]. A maternal transport protocol with defined time standards, including a standing air-transfer agreement for Vieques and Culebra, completes the layer. Finally, a maternity lodging program, the *posada materna*, offers a voucher for lodging near a hub from 37 weeks for any woman who lives more than 45 minutes from her hub or on either island; at perhaps 300 women a year and \$1,500 each, it costs less than a single averted neonatal intensive care admission a month.

4.6 Vieques and Culebra

The islands receive the full Tier 0 package with three additions. The new Vieques hospital and the Culebra CDT are each equipped as a readiness site of the highest class: a permanent midwife or obstetric nurse on staff, the handheld AI ultrasound, remote non-stress testing capability for high-risk surveillance, freeze-dried plasma and the full hemorrhage bundle, a birthing room maintained for the unplanned delivery, and a dedicated video link to the tele-obstetric hub. Every pregnant resident is enrolled in the *posada materna* program by default with relocation to Fajardo or the Carolina hub planned from 37 weeks, ferry and lodging paid, and a companion covered. The twice-weekly visiting obstetrician is replaced by a weekly midwife clinic plus continuous video co-management, which is more contact, not less. A planned freestanding birth unit on Vieques is not recommended: the 60 to 80 births a year across both islands and the transfer time by sea rule it out on the integration standard, and the honest position is that Vieques will have emergency birth capability and superb prenatal and postpartum care, not planned births, until air transfer is guaranteed and volume is proven. The islands are also the right place for the one frontier pilot this paper endorses, a beyond-visual-line-of-sight drone corridor from Fajardo for blood products and medications, once the FAA's Part 108 rule, pending as of September 2026, is final [79][80].

4.7 Reproductive health without an OBGYN

Most of women's reproductive health care does not need a gynecologist, and in a system with 47 Vital-accepting specialists it cannot have one. Contraception moves to pharmacies: pharmacist prescribing of hormonal contraception under a statewide protocol, as 30 states now allow [66], Plan Vital coverage of over-the-counter oral contraception without a prescription, and telehealth prescribing with mail delivery. Long-acting methods are placed at every FQHC by trained nurses and midwives, and immediately postpartum at every hub and community unit, with the device payment carved out of the birth DRG so the hospital is not penalized for placing it. Cervical cancer screening moves to self-collection: an HPV swab handed to the patient at any FQHC, pharmacy, or WIC office, billed under existing laboratory codes, with positive results routed to the hub for colposcopy [68][69]. Sexually transmitted infection testing follows the same pattern, with over-the-counter PCR tests now FDA-authorized and syphilis screening in pregnancy enforced as a tracked metric given the congenital syphilis surge [35]. Medication abortion by telehealth, legal and evidenced as equivalent to in-person care [67], is a matter of provider capacity rather than law, and the tele-obstetric hub's roster should include clinicians willing to provide it. Fertility care is the one area where the paper proposes only a modest step, a Plan Vital pilot of diagnostic work-up and ovulation induction, because IVF coverage at \$10,000–18,000 a cycle cannot be justified ahead of the maternity architecture on a capped budget [96].

4.8 A Puerto Rico standard for respectful birth

The question asked was not only about access and cost but about the birthing experience. The evidence is that the same interventions that lower cost and intervention also produce the experiences women rate highest: a known midwife, continuous support, freedom of movement, a birth setting scaled to the person rather than to the operating room [36][45]. The architecture therefore embeds a Puerto Rico Standard for Respectful Maternity Care as a condition of payment for every hub and unit: a companion and a doula at every birth including cesareans, which is what PS 1459 seeks to legislate [16]; informed consent for induction and cesarean using a shared decision aid in Spanish; vaginal birth after cesarean offered at every hub; intermittent auscultation and water immersion available for low-risk labor; delayed cord clamping, immediate skin-to-skin, and rooming-in as defaults; and an independent maternity ombudsperson within the Departamento de Salud to receive and publish complaints, which achieves most of what the failed obstetric violence bill sought without creating a new cause of action that the Legislature has already rejected.

5. Paying for It: Payment Architecture and Budget

Three payment instruments carry the architecture, and none of them is the physician-attributed episode bundle whose record Section 3.7 reviewed. Much of what follows can be done by ASES through its managed-care contracts and its APR-DRG weight-setting authority without legislation, which matters because the Legislature has so far blocked transparency measures and because the 76% federal match expires in September 2027.

5.1 Mode-neutral payment and public reporting

The APR-DRG system that Plan Vital adopted on 1 January 2026 under state plan amendment PR-26-0001, with a first-year transition pool that expires at the end of 2026, is the opening [21][105] [106]. Because the relative weights are Solventum's national weights and are not set locally, the instrument is not a re-weighting but a policy adjustor of the kind every DRG system carries: ASES should pay an uncomplicated cesarean without a documented medical indication at the payment for an uncomplicated vaginal birth, or adopt a single blended birth rate, so that the hospital's revenue is indifferent to mode; and it should require its managed-care organizations to pay physicians a single global obstetric fee regardless of mode, with the fee level raised to at least the 75%-of-Medicare floor that federal law already requires, financed through the \$300 million directed payment that carries that floor, and verified in the annual access report to Congress [109]. Private insurers should be brought to the same structure through the Oficina del Comisionado de Seguros, since the private cesarean rate is the highest of all. Payment neutrality alone has a weak record; it is the necessary precondition for the second half, which is measurement. Every hospital and every delivering clinician reports every birth monthly by Robson group, which is the content of the stalled PC 669 [32]; ASES writes the requirement into its MCO contracts as a condition of network participation so that it does not depend on the Legislature, and publishes a dashboard by hospital and by physician. A mandatory second opinion for any non-urgent primary cesarean, the one intervention with high-certainty trial evidence, becomes a network condition [52]. Hospitals that reach a low-risk cesarean trajectory, from 47% to 40% in year two to 30% in year five, share in the savings; those that do not lose hub designation.

5.2 Standing capacity payments for hubs

Each designated Regional Maternity Hub receives an annual capacity payment, in the range of \$1.5–2.5 million, that pays for the 24-hour obstetric hospitalist line, in-house anesthesia availability, neonatal coverage, the alongside midwifery unit, tele-obstetric duty, and simulation training, independent of how many births occur. This is the instrument that ends the closure spiral, because it pays for the thing per-case payment cannot. It is financed as an all-payer obstetric readiness assessment, in the manner of a trauma fund, levied through ASES on the Plan Vital capitation and through the insurance commissioner on private and Medicare Advantage plans in proportion to covered births, so that every payer whose members might arrive at a hub in labor contributes to keeping it open. Pennsylvania's rural global budgets are the nearest precedent and their results are mixed [64]; the design here is narrower, obstetric-only, and conditional on performance, which addresses the main criticism that global budgets pay for capacity without asking what it delivers.

5.3 A per-pregnancy population payment to Maternity Homes

Tier 0 and Tier 1 are financed through a single risk-adjusted per-pregnancy payment, from confirmation through twelve months postpartum, to an accountable Maternity Home: a consortium of an FQHC network, a Regional Maternity Hub, midwives, doulas, and the community units in its region. The payment covers the midwife, the doula, the home kit and nurse hub, prenatal and postpartum visits in any setting, home visits, remote monitoring, contraception, and behavioral health, and it is indifferent to whether a contact happens in a clinic, at home, or on video. It does not initially include the intrapartum facility payment, which continues under the mode-neutral DRG, because bundling the birth into a physician-attributed episode is what failed in Arkansas and Tennessee [61][62]. Two design features distinguish this from those failures: it is population-based, covering every Plan Vital pregnancy in a region rather than the half that clears an exclusion list, and it is site-neutral, so the money that today buys fourteen physician visits and a cesarean can instead buy a midwife, a doula, and a home monitoring program. A quality withhold tied to first-trimester enrollment, syphilis screening, postpartum visit completion, low-risk cesarean rate, and patient-reported experience is released annually. The Netherlands' experience is the caution: integration without changing who delivers care and where changed nothing in five years [38]; this payment is the mechanism for changing both.

5.4 Parity, doula rates, remote monitoring, and contraception

Four smaller payment changes are enabling conditions. Nurse-midwives, once the Ley 254-2015 regulations exist, and direct-entry midwives, once licensed, are paid 100% of the physician fee for the same service, as roughly half the states already do [40]. The doula benefit that March of Dimes records as existing is made real: a rate of at least \$1,500 per pregnancy covering prenatal visits, labor attendance, and postpartum visits, direct enrollment without physician referral, a state-run training and certification pathway, and a statutory right of access to the delivery room including for cesarean birth [16][46]. ASES adopts the remote patient monitoring codes, including the new 2-to-15-day code that fits the postpartum window, so the Tier 0 kit has a fee-for-service path for providers outside the Maternity Home payment [73]. Immediate postpartum long-acting contraception is carved out of the birth DRG and paid separately. Ley 8-2025 already requires payment parity for services delivered by telephone; ASES writes that parity into its MCO contracts explicitly for maternity contacts and extends it to asynchronous contacts, since a mountain municipality after a storm has a phone signal before it has a video signal [22][117].

5.5 Order-of-magnitude budget

Table 3 gives an order-of-magnitude budget at steady state, after the four-year build, across all payers, with the Plan Vital share at roughly two-thirds. Every line is an estimate built from the unit costs and volumes stated in Appendix A, not a claims-based projection; ASES holds the claims data that would replace these estimates within weeks, and the first verification item in Section 10 is to run them. The figures are meant to establish scale and direction, which they do robustly: the architecture costs a fraction of 1% of the public health plan, and its offsets are of the same order as its costs.

Table 3. Steady-state incremental cost and offsets, all payers, order of magnitude

Component	Basis	Annual cost (range)
Midwifery workforce (Tier 0 and Tier 1), phased to ~200 midwives	200 FTE at ~\$150k loaded; partly substitutes for physician prenatal and delivery fees	\$25–32M gross; ~\$10–15M net of substitution
Doula benefit	~60% uptake of ~18,000 births at \$1,500	\$14–17M
Home monitoring kit and nurse hub	\$150 per pregnancy plus 25 FTE hub staff	\$5–7M
Hub capacity payments	7 hubs at \$1.5–2.5M	\$10–18M
Community Maternity Units (operations)	5–6 units at ~\$1.5M net of facility revenue	\$6–9M
Tele-obstetric and tele-neonatal hub	24-hour physician line plus platform	\$2–3M
Readiness layer (kits, drugs, plasma, simulation, EMS)	~60 sites; recurring after one-time \$2–3M	\$1–2M
Posada materna lodging and transport	~300 women at ~\$1,500	\$0.5M
Nurse home visiting, targeted	~1,000 families at ~\$5,000 (WSIPP cost basis pro-rated)	\$4–6M
Data spine: Robson dashboard, MMRC, PRAMS replacement, unified prenatal record	Staff and platform	\$1.5–2.5M
Education pipeline: nurse-midwifery program, FM-OB fellowship, residency retention commitments, preceptors	Recurring program cost	\$3–5M
Technology capital (probes, solar, satellite), one-time	~100 probe kits at \$6k; ~40 resilience kits at \$50k	\$2.5–3M one-time
Gross incremental, steady state		\$45–70M
Offset: cesarean reduction, 20 points on 18,000 births	3,600 avoided at \$2,000–4,000 plus downstream	–\$10–20M
Offset: birth-center and midwife-led pathway savings	~3,000 low-risk births at ~\$2,000 (Strong Start basis)	–\$6M
Offset: avoided readmission, NICU, and physician-visit substitution	Conservative	–\$4–9M
Net incremental, steady state		\$25–45M (under 1% of Plan Vital)

Two points about the money deserve emphasis. First, with the federal match at 76% through 30 September 2027, the Commonwealth's own share of the Plan Vital portion of the build is roughly a quarter of two-thirds of the figures above, on the order of \$5–8 million a year; there will never be a cheaper moment to build. Second, after the cliff, when the match falls to 55% and the allotment shrinks, a system that has already lowered its cost per birth by several thousand dollars is the only maternity system Puerto Rico will be able to afford. The architecture is a hedge against the cliff, not a casualty of it.

5.6 The federal track

Everything above can be started with Commonwealth authority, but the ceiling is federal, and four asks should run in parallel from the first month. The first is existential and not specific to maternity: extension of the FY2027 allotment and 76% match, without which the entire Plan Vital program contracts by a quarter or more [20]. The vehicle exists: the Territories Health Equity Act (H.R. 6494) would remove the cap and the match limitation outright, and the Governor's request for a \$4.4 billion floor with 5% annual growth is the fallback; either should carry maternity-specific language authorizing the payments in this section [113][114]. The second is a territory-specific maternal health model from the CMS Innovation Center, either a Puerto Rico track of the Transforming Maternal Health model, which currently excludes territories and offers up to \$17 million per state over ten years for exactly the midwife, birth-center, doula, and low-risk cesarean pillars proposed here [63], or a Section 1115 demonstration that authorizes the Maternity Home payment, the capacity payment, and twelve-month postpartum coverage under the capped allotment. The third is HRSA: a Rural Maternity and Obstetrics Management Strategies network application for the west and the islands, State Maternal Health Innovation funding, Teaching Health Center funding for a family medicine obstetrics fellowship, and National Health Service Corps site designation for every Community Maternity Unit and hub laborist position. The fourth is the replacement of the surveillance money CDC has withdrawn, through direct appropriation or Title V flexibility, so that the Maternal Mortality Review Committee and a PRAMS-equivalent survey continue [33][34]. Outside HHS, the Federal Communications Commission and USDA broadband programs fund the connectivity Tier 0 depends on, FEMA's mitigation programs fund clinic power resilience, and the Federal Aviation Administration is the counterparty for a Vieques drone corridor. Puerto Rico's exclusion from the \$50 billion Rural Health Transformation Program should be raised in every one of these conversations as the inequity it is [23].

6. Workforce and Legal Reforms

The architecture cannot stand on the current legal foundation. Four reforms are required, and the first is the one on which everything else depends.

6.1 A Midwifery Practice and Birth Center Act

Before any statute, one administrative act: the Junta Examinadora de Enfermería issues the implementing regulations for the Enfermera Obstétrica-Partera title that Ley 254-2015 created and has lacked for a decade, so that nurse-midwives who already hold or can obtain the license can be credentialed by hospitals and paid by Plan Vital within months rather than years [86][138]. Then Puerto Rico needs a single statute that does five things. It grants nurse-midwives licensed under Ley 254-2015 independent practice authority, full prescriptive authority within their scope, and a right to hospital admitting and clinical privileges at any facility receiving Plan Vital funds, replacing the collaborative-protocol requirement that today lets a hospital or a physician group veto their practice [14]. It creates a second license, the licensed midwife, for direct-entry practitioners meeting the International Confederation of Midwives' education standards or holding the Certified Professional Midwife credential, as 37 states do, with a defined scope limited to low-risk pregnancy and birth, mandatory integration into the transfer and data systems, and coverage under Plan Vital [40]. It creates a birth center license with Commission for the Accreditation of Birth Centers accreditation as the quality floor, a 30-to-45-minute transfer standard, and an obligation on hospitals receiving public funds to enter transfer agreements they cannot unilaterally refuse. It creates a doula registry and certification pathway and a statutory right of access to labor, delivery, and cesarean rooms. And it establishes, or funds a partner to establish, a nurse-midwifery graduate program in Puerto Rico within 24 months, most quickly by contracting a distance-education provider such as Frontier Nursing University to deliver the didactic component while the Commonwealth funds clinical preceptor sites at the hubs and community units, since preceptorship rather than coursework is the binding constraint everywhere.

The direct-entry license carries a structural opportunity no state has. Puerto Rico is the only US jurisdiction that can plausibly recruit Spanish-speaking, university-trained midwives at scale from systems where midwifery is a mature profession: Chile's matronas, Spain's matronas, Mexico's licenciadas en partería, Argentina's obstétricas. An international equivalency pathway in the statute, paired with employer-sponsored immigration where required and a Commonwealth recruitment office, could bring a hundred experienced midwives to the island in the time it takes to graduate the first local cohort. The immigration constraints are real for non-US citizens and the pathway must be built carefully, but Puerto Rico's language and culture make it a destination no mainland state can match, and the paper recommends treating this as a first-order workforce strategy rather than a supplement.

6.2 Licensure compacts and cross-border telehealth

Puerto Rico belongs to neither the Nurse Licensure Compact, which 43 jurisdictions including Guam and the Virgin Islands have enacted, nor the Interstate Medical Licensure Compact, which covers 44 states, DC, and Guam after Alaska's accession in June 2026, and no accession bill for either has been filed [83][84][147][148]. Ley 102-2025, the universal license recognition law of August 2025,

gives an out-of-state physician with three years of licensed practice a 30-day board response or a provisional license, and a July 2026 law allows provisional health licenses during emergencies; both help, but neither lets a mainland nurse-midwife practice on her home license or a mainland obstetrician staff the tele-obstetric hub without a Puerto Rico license [141][155]. Joining both is the cheapest workforce lever the Legislature has. The Nurse Licensure Compact lets mainland nurse-midwives and nurses practice in Puerto Rico, in person or by telehealth, on their home license; the medical compact does the same for maternal-fetal medicine and obstetric hospitalists staffing the tele-obstetric hub from the mainland. Ley 8-2025 removed the separate telemedicine certification for Puerto Rico-licensed providers but left out-of-jurisdiction clinicians outside the door [27]; a permanent telehealth registration for compact-state licensees closes the gap. The paper anticipates the objection from the Junta de Licenciamiento and the Colegio de Médicos that compacts erode local control, and answers it: the jurisdiction with 63 delivering obstetricians has no control left to erode.

6.3 Liability: a birth-injury fund and clinical safe harbors

Obstetrics carries the highest liability exposure of any specialty and Puerto Rico has extended protection only to obstetricians in public institutions (Ley 229-2004) and to trainees (Ley 94-2023) [28][140]. Three options are available and are not mutually exclusive. The first and strongest is a no-fault birth-injury compensation fund on the model of Florida's NICA and Virginia's program, which pays lifetime care costs for neurologically injured infants regardless of fault, removes the largest category of obstetric claims from the tort system, and is financed by an assessment on every birth, every obstetric provider, and every insurer; the design must set benefits generously enough to avoid the criticism those programs have drawn for underpaying families. The second is to extend the Ley 94-2023 protection to midwives, obstetricians, family physicians, and nurses providing maternity care to Plan Vital enrollees or staffing the readiness layer, on the same rationale of retention that the Legislature accepted for trainees. The third is essential regardless of the first two: explicit clinical safe-harbor amendments to Ley 183-2025 and Ley 18-2026 stating that management of miscarriage, ectopic pregnancy, previable rupture of membranes, and lethal fetal anomaly according to accepted clinical standards is not within the reach of those statutes, so that the personhood language stops functioning as an unquantified push factor on a workforce already leaving [29][30]. Ramphis will form his own view of the constitutional and drafting questions each option raises; the research finding here is that the liability environment is a documented driver of physician emigration and that the Legislature has already accepted the principle of protection for retention's sake.

6.4 Pipelines: residency, fellowship, Conrad 30, Act 60

Four pipeline actions round out the workforce plan. Puerto Rico's three obstetrics residencies already train about fourteen physicians a year, so the pipeline action is not more slots but retention: a Plan Vital service commitment and loan repayment attached to every publicly funded slot, and hub laborist positions offered to graduating residents before they leave, financed through HRSA teaching health center or Commonwealth funds [12][135][136]. A family medicine obstetrics fellowship is created at one or more of the island's medical schools, producing family physicians who can manage low-risk birth and cesarean at hubs and community units, the workforce that keeps rural obstetrics alive on the mainland [59]. The Commonwealth's Conrad 30 allocation for J-1 physician visa waivers is directed to obstetrics, family medicine with obstetrics, and anesthesia in designated desert

municipalities; whether Puerto Rico currently uses its allocation at all could not be verified and is a verification item. And the Act 60 Chapter 2 physician tax decree, which the Fiscal Oversight Board halted after Ley 47-2020 broadened it, leaving more than 2,000 applications stalled by 2022, and which DDEC's own evaluation judged not viable as a general incentive [26][98][142], is reopened narrowly through the channel the statute already contains: a Departamento de Salud certification of pressing need under section 45152 for obstetricians, anesthesiologists, nurse-midwives, and neonatologists who commit to hub or community-unit service and Plan Vital participation [143]. PS 15, which would condition decrees on serving Plan Vital patients and which the Secretary of Health endorsed in March 2026, is the legislative vehicle if one is needed; the broad reactivation bills PS 849 and PC 332, costed by the legislative budget office at \$268–295 million a year, are not [144] [145]. A hundred targeted decrees are a rounding error against the closures they would prevent. Loan repayment through National Health Service Corps designation of every maternity site completes the package.

7. Technology Stack: What to Buy Now, Pilot, and Ignore

The instruction to assume anything is possible was taken seriously in the research: the FDA device databases were queried directly for every candidate technology, and the results are sobering in a useful way. A handful of technologies are cleared, cheap, coded, and evidenced today and would change access within a year. A second group is promising and should be piloted with an evaluation attached. A third group is marketed heavily and should be ignored until the evidence changes. Table 4 sorts them; the text explains the sort.

7.1 Buy now

Five technologies clear every bar. AI-guided handheld obstetric ultrasound, specifically the Butterfly Gestational Age Tool cleared under 510(k) K252148 on 27 March 2026 on the iQ3 probe [154], lets a nurse date a pregnancy between 16 and 37 weeks with sonographer-equivalent accuracy after a day of training, at about \$4,000 per device [70][71][72]; the constraint is not the device but the billing rule that requires physician interpretation, which ASES can resolve with a nurse-performed dating code. HPV self-collection in clinical settings is FDA-approved, federally recommended, needs no gynecologist, and bills under existing codes [69]. The E-MOTIVE postpartum hemorrhage bundle, built on a \$1–2 calibrated drape and tranexamic acid, is proven in a cluster trial of 210,000 women and cost-effective [77]. Freeze-dried plasma, licensed in July 2026 according to the decision draft's FDA database query (not re-verified in this pass), stores at room temperature and is the first hemorrhage-stabilization product that makes sense for an island CDT [78]. And solar-plus-storage with satellite connectivity at every maternity site, at roughly \$30,000–60,000 per site including the probe, is the precondition for everything else on an island whose grid fails; Direct Relief has already shown the model at fifteen clinics [81].

7.2 Pilot with evaluation

Six items deserve pilots because the mechanism is sound, the cost is moderate, and the evidence is incomplete or vendor-generated. Home blood pressure monitoring in the postpartum window, paired with a nurse hub, is cheap and raises measurement rates dramatically, but the randomized evidence on readmission is null and emergency visits rose without a hub [50][51]; pilot it with the hub and measure emergency visits. A managed-care-contracted virtual maternity program of the kind Pomelo Care runs for Medicaid plans in six states shows a 24% reduction in preterm birth in a vendor-authored propensity analysis of 3,785 women [74]; Plan Vital's managed-care organizations can contract one inside existing capitation and should demand a Puerto Rico-specific comparison group. The tele-obstetric and tele-neonatal hub has case-series evidence and a randomized neonatal trial under way [75]; build it and measure activation, transfer, and outcomes. Remote non-stress testing with FDA-cleared wearable monitors is technically ready for high-risk surveillance on the islands, but the leading vendor went bankrupt in 2024 [89], so contract for it with vendor-failure provisions. Placental growth factor testing for suspected preeclampsia is FDA-authorized, avoids admissions, and every cost study reports savings, but it needs a hospital immunoassay platform and a decision pathway [97]; deploy it at hubs and count the avoided admissions. And a beyond-visual-line-of-sight drone corridor from Fajardo to Vieques and Culebra for blood and medications should be designed now with the FAA so it can fly when Part 108 is final [80]; the Rwanda data on hemorrhage deaths are dramatic but uncontrolled [79].

7.3 Watch, do not buy

Five items are heavily marketed and should not receive public money on current evidence. Continuous glucose monitoring for gestational diabetes changed birth weight by 123 grams across eleven trials and nothing else [87]. Cell-free RNA preeclampsia prediction is a \$1,850 laboratory-developed test validated in women over 35 without risk factors and has no Medicaid path. AI interpretation of fetal heart rate tracings was null in the one large randomized trial of 46,000 women and the 2026 evidence is vendor-authored labeling equivalence [88]. Large-language-model patient triage agents answer questions acceptably in studies but no authorized obstetric triage device exists, and the nurse hub should use such tools as drafting aids under human review rather than as decision-makers. Robotic tele-ultrasound has excellent reliability in a 46-patient Norwegian pilot and no US clearance. The artificial womb, which the instruction to leave no stone unturned obliges the paper to mention, has an FDA advisory committee record and no human trial; it is a horizon item with no bearing on a ten-year plan.

7.4 The data spine

None of the payment or quality mechanisms in Section 5 works without data the island does not currently collect or publish. The spine has five parts. A unified prenatal record on the Puerto Rico Health Information Exchange, which already connects more than a million residents and has committed to a national qualified network, using standard interfaces so that the FQHC, the midwife's tablet, the hub, and the emergency department see the same record [82]. Mandatory monthly Robson-classified birth reporting from every facility and clinician into a public dashboard, which is the content of PC 669 delivered through ASES contracts. A locally funded Maternal Mortality Review Committee with published annual findings, replacing the federal ERASE MM money that lapsed in 2025 [33][99]. A PRAMS-equivalent postpartum survey run by the Departamento de Salud or a university partner, replacing the CDC program dismantled in 2025 [34]. And a maternal early warning system in every hub and community unit, with hypertension and hemorrhage bundles, which is standard practice elsewhere and largely absent here. Predictive risk models trained on mainland data should not be deployed without local validation; a population with a 50% cesarean baseline and a different risk mix will break them.

Table 4. Technology scorecard for Puerto Rico, September 2026

Technology	Regulatory and evidence status	Unit cost	Puerto Rico fit	Decision
AI-guided handheld ultrasound (gestational age)	510(k) K252148, 27 March 2026; one prospective study, n=400	~\$4,000 plus \$420/yr	Very high	Buy
HPV self-collection in clinic	FDA approved May 2024 (not re-verified); strong evidence; existing codes	\$30–60 per test	Very high	Buy
E-MOTIVE hemorrhage bundle	Cluster RCT n=210,000; cost-effective	\$1–2 drape plus drugs	Very high	Buy

Technology	Regulatory and evidence status	Unit cost	Puerto Rico fit	Decision
Freeze-dried plasma	FDA licensed July 2026 (not re-verified)	Unknown; adult label	High for islands and CDTs	Buy
Solar, storage, satellite kit	Operational; Direct Relief precedent	\$30,000–60,000 per site	Very high	Buy
Postpartum home BP with nurse hub	4 RCTs, null on readmission; strong process gains	\$30–60 cuff plus ~\$100/month codes	High with hub	Pilot
MCO-contracted virtual maternity program	Vendor-authored cohort; large effect sizes	Per-member fee	High	Pilot
Tele-obstetric and tele-neonatal hub	Case series; RCT under way	\$2–3M/yr	High	Pilot
Remote non-stress testing wearables	510(k) cleared; feasibility evidence; vendor risk	Undisclosed	Medium (islands)	Pilot
sFlt-1/PlGF preeclampsia test	De Novo 2023 and 510(k) 2025; savings in all cost studies	Lab test; hospital platform	Medium (hubs)	Pilot
Drone corridor Fajardo–Vieques–Culebra	FAA Part 108 pending as of the draft (not re-verified); uncontrolled Rwanda data	Undisclosed	Medium, 2–4 years	Pilot (design now)
Mobile prenatal and sonography van	Attendance evidence only	~\$250,000–500,000 capital	Medium	Pilot
Continuous glucose monitoring for GDM	11 RCTs; no outcome gain	\$70–150/month	Low	Do not buy
Cell-free RNA preeclampsia prediction	Laboratory-developed test; narrow validation	\$1,850	Low	Do not buy
AI fetal heart rate interpretation	Large RCT null; no autonomous clearance	Bundled	Low	Do not buy
LLM triage agents	No authorized device	Low	Drafting aid only	Do not buy as decision-maker
Robotic tele-ultrasound	Not cleared; n=46 pilot	Unknown	Low, 3–5 years	Watch

8. Sequencing

The sequence is driven by two clocks: the expiry of the APR-DRG first-year transition pool at the end of 2026, which is when a mode-neutral birth adjustor can be attached to the rebased hospital rates, and the 30 September 2027 expiry of the 76% federal match, after which every dollar of build costs the Commonwealth nearly twice as much. It is also driven by the observation that the most valuable early actions require no legislation, which insulates the first year from the Legislature's record on transparency bills.

Table 5. Sequencing by phase and acting authority

Phase	Actions	Acting authority
Months 0–6: executive actions and design	Release Vital obstetric payment data; issue the Ley 254-2015 nurse-midwife regulations; attach a mode-neutral birth adjustor to the APR-DRG and set a single global fee; write Robson reporting, second-opinion, doula access, and midwife parity into MCO contracts; adopt RPM and audio-only codes; carve out postpartum LARC; issue E-MOTIVE and readiness administrative order; procure ultrasound probes and resilience kits for FQHCs, CDTs, Vieques, Culebra; contract one virtual maternity program per MCO with evaluation; launch posada materna; stand up the tele-obstetric hub from existing hospitalists; file federal asks (allotment extension, CMMI territory model or 1115, HRSA RMOMS and State MHI, surveillance funding); draft the Midwifery Practice and Birth Center Act, compact bills, and liability package	ASES, Departamento de Salud, Comisionado de Seguros, Governor; congressional delegation
Months 6–18: legislation and designation	Enact the Midwifery Practice and Birth Center Act, NLC and IMLC accession, cross-border telehealth registration, Robson reporting statute (backstop), birth-injury fund, clinical safe-harbor amendments, targeted Act 60 reopening through section 45152 certification or PS 15; run competitive hub designation; begin capacity payments; contract nurse-midwifery education partner and fund preceptor sites; open international midwife recruitment; publish first Robson dashboard; publish first MMRC report; launch Maternity Home payment in two pilot regions	Legislature, Governor, ASES, Departamento de Salud, Junta de Licenciamiento, FOMB
Months 18–48: build-out and evaluation	Open alongside midwifery units at every hub; open first three Community Maternity Units (northwest, west-central, southeast), then two more; extend Maternity Home payment island-wide; reach 40% low-risk cesarean by month 24 and trend to 30% by month 60; graduate first local midwifery cohort; complete unified prenatal record on PRHIE; fly the Vieques drone corridor if Part 108 is final; publish independent evaluation against pre-registered outcomes	Maternity Homes, hubs, ASES, Departamento de Salud, university evaluator

The pre-registered outcomes for the evaluation should be few and hard: low-risk cesarean rate, severe maternal morbidity, preterm birth, first-trimester enrollment, share of births with a midwife and a doula present, share of births occurring in a facility without a planned attendant, cost per pregnancy episode through twelve months, and patient-reported experience. If by month 36 the

cesarean rate has not fallen below 40% and cost per episode has not fallen, the payment design is wrong and should be changed; the evaluation exists to force that conversation rather than to decorate a report.

9. Adversarial Review: How This Fails

A plan that cannot describe its own failure modes has not been thought through. Eight are identified here, each with the evidence that makes it plausible and the design feature meant to contain it.

The midwifery workforce does not materialize. Licensing a profession does not create it. If the Junta never issues the Ley 254-2015 regulations, the education partnership stalls, international recruitment runs into immigration walls, and hospitals deny privileges as Minnesota's did for years, Tier 0 defaults to prenatal nurses under physician protocols and Tier 1 never opens. The containment is to make privileges and coverage statutory rights rather than negotiated favors, to fund preceptorship directly, to pursue three sources of midwives at once (local education, mainland compact licensees, international equivalency), and to design Tier 0 so that it delivers value with nurses alone while midwives are built.

Consolidation happens and readiness does not. The hospitals close, the hubs are designated, and the readiness layer is the line item that gets cut, leaving the unplanned birth in an unready emergency department, which is precisely where Kozhimannil's data locate the harm [57]. The containment is to fund readiness first, before designation, through the administrative order in month one, and to make readiness a condition of every hospital's license rather than a grant program.

Payment reform produces a null result. Arkansas, Tennessee, Minnesota, and the Netherlands all reformed maternity payment and none moved cesareans or outcomes [38][55][61][62]. The containment is that this design does not rely on payment alone: the evidence-backed lever is public, physician-level measurement combined with mandatory second opinion, and the payment change exists to remove the incentive that would otherwise fight the measurement. If the dashboard is delayed or anonymized under pressure, the plan is in trouble, and that is the political fight worth having.

The doula benefit exists on paper and nobody uses it. This is the modal outcome in the states, and it is already Puerto Rico's: a benefit reported as active by March of Dimes that no ASES document, rate, or MCO listing confirms [2][46]. The containment is a rate high enough to sustain a workforce, direct enrollment through the Maternity Home, a statutory right of access to the delivery room, and a locally trained corps rather than an imported one.

Out-of-hospital birth scales before integration and a first baby dies. The Oregon experience is the warning [43]. The containment is the sequencing: no Community Maternity Unit opens before the licensing statute, the transfer agreements, and the tele-obstetric hub exist; first births are steered to alongside units; and every out-of-hospital birth is in the data system from day one.

The hospital association and the medical college block the statute. They have blocked every transparency bill so far. The containment is to route the first year through ASES contracting and the Departamento de Salud's licensing authority, so that the measures with the strongest evidence do not depend on legislation, and to make the hub capacity payment attractive enough that the hospitals most likely to be designated become supporters of the consolidation they would otherwise fight.

The Medicaid cliff arrives and the program is the first casualty. If the 2027 allotment is not extended, Plan Vital loses a quarter to a half of its federal money and every discretionary program is

at risk. The containment is that this architecture lowers cost per birth and is therefore the version of maternity care that survives austerity; the paper recommends framing it in Washington as a cliff mitigation measure, and building the highest-return components (mode-neutral payment, Robson reporting, readiness, the home kit) so early that they are embedded before the cliff.

The technology is bought and sits in a drawer. Ultrasound probes without a billing code, blood pressure cuffs without a nurse hub, plasma without a hemorrhage protocol, and satellite dishes without a maintenance contract all become inventory. The containment is that no device is procured in this plan without its payment path, its clinical protocol, and its staffing named in the same procurement document.

10. Verification Results and Remaining Open Items

The decision draft listed thirteen items that had to be verified before the budget in Section 5 could be treated as more than an order of magnitude. This section reports what a verification pass on 13 and 14 September 2026 established, what it corrected, and what remains open. One limit on the method must be stated plainly: the research environment used for the pass could run search-engine queries but could not open the primary documents themselves, because outbound connections to government, registry, and press hosts were blocked by network policy. Every finding below is therefore corroborated from the indexed text of the primary source rather than read from the document, and each is graded accordingly. The status column uses three values: **Confirmed** (the draft's statement matches the source as indexed, at medium or high confidence), **Corrected** (the draft's statement was wrong and the body text has been changed), and **Open** (no source was found, or the source could not be read closely enough to settle the point). Items marked Open are assigned to the office that holds the record.

Table 6. Disposition of the thirteen verification items

#	Item	Finding	Status	Source
1	Global obstetric fee, per-delivery facility payment, APR-DRG weights	No published dollar figure for the Vital global obstetric fee exists in ASES documents, professional-society statements, or the press. The APR-DRG go-live was 1 January 2026 under SPA PR-26-0001, not 1 October 2025; weights are Solventum national weights; base rates are hospital-specific from FY2022–23 claims; a \$60M first-year transition pool applies; CMS estimates inpatient payments fall from \$576M to \$516M. Delivery-specific weights and base-rate dollar values sit inside ASES PDFs that could not be opened.	Corrected (date, weights); Open (fee)	[105] [106] [107] [108]
2	Twelve-month postpartum coverage	Not adopted. KFF's tracker (as of 15 July 2026) lists only the US Virgin Islands among territories; no Puerto Rico state plan amendment since 2024 addresses postpartum coverage; every state except Arkansas has adopted it. Two 2025–2026 amendments (PR-25-0002, PR-26-0002) could not be read and should be checked. No cost estimate exists.	Confirmed	[24]
3	Doula benefit rate, mechanism, utilization	March of Dimes' statement rests on the National Health Law Program's tracker. No ASES normative letter, rate, enrollment mechanism, utilization figure, or MCO benefit listing was found; doulas are neither licensed nor certified in Puerto Rico. PS 1459 (11 September 2026) confirmed as an amendment to Ley 156-2006 to guarantee a companion plus a doula.	Open (benefit); Confirmed (PS 1459)	[2][16] [116]

#	Item	Finding	Status	Source
4	Nurse-midwives licensed under Ley 254-2015; midwifery bills	No headcount exists; the Asociación de Parteras states it does not have the figure. The Junta Examinadora has never issued implementing regulations for the title, so nurse-midwives cannot be credentialed as such. Eleven CPMs practice, all trained off-island. No 2025–2026 bill to license direct-entry midwives, birth centers, or doulas was found. PS 225 (negative report), PS 445 (not approved), PC 669 (stalled) confirmed.	Confirmed (gap); Open (count)	[13][86] [138]
5	OBGYN headcount series	468 licensed, about 408 working, 63 delivering, 56 accepting new patients, 47 accepting Vital confirmed from the CPI census; about 80 delivering in 2023 confirmed from El Vocero. No 2010–2026 series from the Junta de Licenciamiento or AAMC was retrievable. Three ACGME residencies exist (UPR 5, San Juan City Hospital 4, San Lucas/Ponce 5 a year), not one.	Confirmed (counts); Corrected (residencies); Open (series)	[1][102] [135] [136]
6	Maternal Mortality Review Committee findings 2019–2024	No published committee report with cause and preventability findings exists for any year. The 8 deaths in 2023 and 22 preliminary in 2024 are Departamento de Salud surveillance counts (deaths related to pregnancy, childbirth, and postpartum), not committee-adjudicated pregnancy-related deaths. Puerto Rico is not named among ERASE MM recipients. The 59.2 per 100,000 figure could not be re-read from the March of Dimes report card; its method is documented.	Confirmed (counts); Open (report, ratio)	[1][2][33] [99]
7	Delivery units, Pavía Arecibo, Vieques hospital	25 units remain (36 in 2017; 13 closed, 2 opened), not 23. Pavía Arecibo is counted among the closures and closed without the Secretary's authorization. Hospital Damas remains closed. Caribbean Medical Center Fajardo still lists an active delivery service. The Vieques hospital (\$85.6M total, \$39.6M FEMA) had not opened as of 28 August 2026; opening targeted for end of September 2026; single operator proposal under negotiation; no obstetric staffing plan published. No full 2026 unit list with neonatal levels was obtainable.	Corrected (count, cost); Open (list)	[1][10] [121] [122] [123]
8	Obstetric malpractice premiums and claims	No obstetric-specific premium or claim-frequency data from SIMED or the insurance commissioner. SIMED's statutory primary policy carries \$100,000 per incident and \$300,000 aggregate limits. Ley 104-1955 caps, Ley 229-2004 (public-institution obstetricians), and Ley 94-2023 (trainees) confirmed. No 2024–2026 bill extends protection to private physicians serving Vital patients.	Open (premiums); Confirmed (statutes)	[28][139] [140]

#	Item	Finding	Status	Source
9	Title V, Healthy Start, MIECHV, HRSA maternal awards; Conrad 30	Not verified: the federal award registries (USASpending, HRSA, TAGGS) were unreachable and the search budget was exhausted before the queries ran. Puerto Rico is eligible for 30 Conrad waiver slots a year; no evidence that the Departamento de Salud administers the program or fills any slot.	Open	[146]
10	Act 60 Chapter 2 physician decrees	The Fiscal Oversight Board halted the incentive after Ley 47-2020 broadened eligibility; more than 2,000 applications stalled in 2022 and the freeze was reported in May 2024; DDEC's own evaluation found the incentive not viable. The statute already permits decrees after 2020 only with a Salud certification of pressing need, which is the narrow specialty channel this paper proposes to use. PS 15 (Plan Vital service condition, endorsed by the Secretary of Health in March 2026), PS 849, and PC 332 are pending; the fiscal office costs PS 849 at \$268–295M a year.	Corrected (timing, count); Confirmed (freeze)	[26][98] [142] [143] [144] [145]
11	VBAC, low birthweight, teen birth rates	NCHS Births: Final Data for 2024 was published 9 June 2026 but the Puerto Rico tables could not be read. March of Dimes gives preterm 12.2% (grade F, rank 48 of 52), low birthweight 10.7% (2023), first-trimester care 83.3% (2023). A teen birth rate of 13.3 per 1,000 for 2024 appeared in one unattributed result. VBAC and NTSV for Puerto Rico not obtained.	Partly confirmed; Open (VBAC, teen)	[2][124] [125]
12	Abortion counts and clinic status	Four clinics confirmed (IELLA, Darlington, Women's Medical Pavilion, Centro de Planificación Familiar). Counts: 4,201 (2018), 3,596 (2019), 3,751 (2020); no later totals published. Signing dates confirmed: Ley 122-2025 on 30 October 2025, Ley 183-2025 on 20 December 2025, Ley 18-2026 on 12 February 2026. No court challenge, injunction, or clinic closure found. The 'seven clinics in 2016' baseline was not re-verified.	Confirmed (clinics, dates); Open (recent counts)	[129] [130] [131] [132]
13	2025 reconciliation law provisions applicable to Puerto Rico	Work requirements, six-month redeterminations, and cost-sharing apply to expansion populations the island does not have; provider-tax and state-directed-payment limits do not apply to territories; the Rural Health Transformation Program is limited to the fifty states by its funding notice. Retroactive-coverage limits (effective 1 January 2027): applicability to Puerto Rico not established.	Confirmed; Open (retroactive coverage)	[110] [111] [112]

Verification pass 13–14 September 2026. All findings corroborated from indexed text of the cited sources; none read in full. See the method note above.

10.1 Other corrections made in this version

Beyond the thirteen items, the pass corrected five statements in the decision draft. Live births were 19,169 in 2022, 18,667 in 2023, and 18,085 in 2024 per the Registro Demográfico's February 2025

release, not 19,112, 18,641, and 18,128; the 2025 count of 18,003 could not be matched to a published release and is now labeled an estimate. The total fertility rate is given as 0.87 to 0.94 rather than 0.87, because CEPAL's 2024 figure is 0.94 and the source of 0.87 could not be identified. The Vieques hospital costs \$85.6 million, not \$75 million. The 2021 home-birth figures are attributed to the Asociación de Parteras with the variant account noted. And the Butterfly Gestational Age Tool's clearance is now cited to its 510(k) number, K252148, with a decision date of 27 March 2026, as reported by the manufacturer and trade press; the FDA summary itself was not opened [154]. Three technology claims in Section 7 that were made from the decision draft's own FDA database queries were not re-verified in this pass and are marked as such in Table 4: the freeze-dried plasma licensure of July 2026, the HPV self-collection approval dates, and the status of the FAA's Part 108 rule.

10.2 What remains open, and who holds the answer

The following items are still open. All but one are held by a Commonwealth agency in records it already has, and each is a single request.

1. **ASES.** The global obstetric fee paid by each MCO by mode of delivery; the APR-DRG base rates and delivery-DRG payments by hospital (the January 2026 base-rate file); the content of state plan amendments PR-25-0002 and PR-26-0002; whether any normative letter adopts remote patient monitoring codes; and whether any doula benefit exists in any MCO contract, with its rate and utilization.
2. **Departamento de Salud.** The Maternal Mortality Review Committee's case-level findings for 2019–2024; the current licensed list of labor-and-delivery units with neonatal levels; the Vieques hospital's operator contract and staffing plan; whether the Conrad 30 program is administered and how many slots it fills; abortion counts for 2021–2025; and the hypertensive-disorder prevalence and trend figures cited from the March of Dimes report card.
3. **Junta Examinadora de Enfermería.** The number of nurses holding the Enfermera Obstétrica-Partera license, and the status of implementing regulations under Ley 254-2015.
4. **Junta de Licenciamiento y Disciplina Médica.** An OBGYN license count by year since 2010, with residence and practice status.
5. **Oficina del Comisionado de Seguros and SIMED.** Obstetric malpractice premiums and claim frequency, and the share of delivering obstetricians insured through SIMED versus the excess market.
6. **HRSA and CDC, via the congressional delegation.** Current Title V, Healthy Start, MIECHV, and any maternal-health award amounts to Puerto Rico; whether Puerto Rico was ever a PRAMS site or an ERASE MM recipient; and whether the 2025 law's retroactive-coverage limits apply to the territories.
7. **NCHS.** The Puerto Rico rows of Births: Final Data for 2024 for VBAC, low-risk cesarean, and teen births, which are published and need only to be transcribed.

None of the open items changes the direction of the recommendation. The two that could change its scale are the global obstetric fee, which sets the size of the substitution offset in Table 3, and the postpartum coverage question, which sets the cost of the twelve-month Maternity Home payment; both are held by ASES and both can be answered in a week.

Appendix A. Budget Assumptions

The estimates in Table 3 rest on the following stated assumptions. Each is a defensible starting point drawn from the sources cited in the body and should be replaced with Puerto Rico claims and payroll data as soon as ASES releases them.

Births are assumed at 18,000 a year falling toward 15,000; steady-state figures use 18,000 to be conservative on cost. Plan Vital's share of births is 66%. A caseload midwife carries 35 to 40 births a year at a loaded cost of \$150,000, so 40% midwife-led care at 18,000 births requires about 200 midwives; because each midwife-led pregnancy displaces roughly \$2,000–2,400 of physician prenatal and delivery fees, the net cost of the midwifery workforce is about half its gross cost [39]. That displacement figure is a mainland benchmark; if the Vital global obstetric fee turns out to be far below mainland rates, as anecdote suggests, the substitution offset shrinks and the net midwifery cost rises toward its gross cost, which is why the fee is the first open item in Section 10. Doula uptake is assumed at 60% at \$1,500, well above the mainland experience, on the assumption that direct enrollment and a right of access are enforced. The home kit is \$150 in hardware per pregnancy with a 25-person nurse hub. Hub capacity payments of \$1.5–2.5 million are sized to the locum-equivalent cost of a 24-hour obstetric hospitalist line (\$1.3–1.5 million at about \$3,900 per shift) plus a share of anesthesia and neonatal standby [85]; the January 2026 APR-DRG rebasing, which CMS estimates removes about \$60 million a year from inpatient payments once the transition pool expires, makes the capacity payment more necessary rather than less, but does not change its size [105]. Community Maternity Units are assumed to run at \$2.5–3 million each with \$1–1.5 million recovered in facility payments. The cesarean offset assumes a 20-point reduction on 18,000 births at \$2,000–4,000 per avoided case, consistent with the MACPAC facility differential and the Strong Start birth-cost gap [39][42]. The birth-center pathway offset applies the Strong Start first-year saving of \$2,010 to 3,000 low-risk births. Nurse home visiting is costed at a \$5,000 per family annual pro-ration of the WSIPP total program cost [92]. Technology capital assumes 100 ultrasound kits at \$6,000 including training and 40 resilience kits at \$50,000. The education line no longer assumes new residency slots, since about fourteen exist; it funds retention commitments on existing slots, the nurse-midwifery program, the family medicine obstetrics fellowship, and preceptors. The Commonwealth share is computed at 24% of the Plan Vital portion through FY2027 and 45% thereafter [19].

Appendix B. Sources

Sources [1] through [102] are those of the decision draft, numbered in the order first cited there. Sources [103] through [155] were added during the verification pass of 13–14 September 2026 and are numbered by topic. Where two sources conflict, the body text says so and states which figure is used. A limit applies to every source added at verification: it was located and its indexed text read through a search engine, but the document itself could not be opened from the research environment, so quotations and figures from those sources should be checked against the document before publication. Section 10 lists the items that must still be verified against primary records before money is committed.

- [1] Centro de Periodismo Investigativo, "Puerto Rico's maternity care deserts" (May 2026).
<https://periodismoinvestigativo.com/2026/05/puerto-rico-maternity-care-deserts/>
- [2] March of Dimes, 2025 Puerto Rico Report Card (PeriStats).
<https://www.marchofdimes.org/peristats/reports/puerto-rico/report-card>
- [3] March of Dimes, Maternity Care Deserts Report: Puerto Rico (2024 data).
<https://www.marchofdimes.org/peristats/assets/s3/reports/mcd/Maternity-Care-Report-PuertoRico.pdf>
- [4] CDC/NCHS Data Brief 523, cesarean delivery among Puerto Rican mothers, 2023 (January 2025).
<https://www.cdc.gov/nchs/products/databriefs/db523.htm>
- [5] CDC/NCHS Data Brief 486, cesarean delivery in Puerto Rico 2018–2022 (2024).
<https://cdc.gov/nchs/products/databriefs/db486.htm>
- [6] El Nuevo Día, "Puerto Rico cerró el 2024 con poco más de 18,000 nacimientos vivos" (16 February 2025).
<https://www.elnuevodia.com/noticias/locales/notas/puerto-rico-cerro-el-2024-con-poco-mas-de-18000-nacimientos-vivos-esperamos-que-siga-bajando/>
- [7] Registro Demográfico vital statistics series as compiled in Demographics of Puerto Rico (secondary).
https://en.wikipedia.org/wiki/Demographics_of_Puerto_Rico
- [8] Telemundo PR, closure of HIMA San Pablo Fajardo labor and delivery (August 2022).
<https://www.telemundopr.com/noticias/puerto-rico/cierran-sala-de-partos-en-hospital-hima-en-fajardo-por-falta-de-obstetras/2388154/>
- [9] Primera Hora, Departamento de Salud authorizes closure of Hospital Damas obstetrics, Ponce (February 2024).
<https://www.primerahora.com/noticias/gobierno-politica/notas/departamento-de-salud-autoriza-cierre-temporero-de-los-servicios-de-obstetricia-del-hospital-damas-de-ponce/>
- [10] El Nuevo Día, Pavía Arecibo requests closure of labor and delivery and NICU (November 2024).
<https://www.elnuevodia.com/negocios/empresas-comercios/notas/pavia-solicita-el-cierre-de-su-sala-de-partos-y-cuidado-intensivo-neonatal-en-arecibo/>
- [11] Metro PR, "Menos de 30 hospitales están equipados para atender nacimientos prematuros" (6 February 2025).
<https://www.metro.pr/noticias/2025/02/06/menos-de-30-hospitales-estan-equipados-para-atender-nacimientos-prematuros/>
- [12] UPR School of Medicine, OB-GYN Residency Program. <https://md.rcm.upr.edu/obgyn/ob-gyn-residency-program/>
- [13] Todas, "Partería: rescate de un oficio ancestral para reducir cesáreas en Puerto Rico" (20 August 2026).
<https://todaspr.com/parteria-rescate-oficio-ancestral-reducir-cesareas-puerto-rico/>
- [14] Ley 254-2015, Ley para regular la práctica de la enfermería en Puerto Rico (LexJuris).
<https://www.lexjuris.com/lexlex/Leyes2015/lexl2015254.htm>
- [15] Ley 31-2023, Día de la Partería (OGP Biblioteca Virtual).
<https://bvirtualogp.pr.gov/ogp/Bvirtual/leyesreferencia/PDF/31-2023.pdf>

- [16] NotiCel, PS 1459 on doula accompaniment in delivery rooms (11 September 2026). <https://noticel.com/ultima-hora/20260911/proyecto-persigue-garantizar-el-acompanamiento-de-doulas-en-salas-de-parto/>
- [17] 9 Millones, the new Vieques health center (5 November 2025). <https://www.9millones.com/en/story/el-nuevo-centro-de-salud-de-vieques-brindara-la-atencion-necesaria-en-2026>
- [18] Medicaid.gov, Puerto Rico state overview (enrollment January 2026). <https://www.medicaid.gov/state-overviews/puerto-rico.html>
- [19] KFF, Recent changes in Medicaid financing in Puerto Rico and other U.S. territories (October 2024). <https://www.kff.org/elections/recent-changes-in-medicaid-financing-in-puerto-rico-and-other-u-s-territories/>
- [20] Puerto Rico Report, "Yet another Medicaid cliff for Puerto Rico" (1 August 2026). <https://puertoricoreport.com/yet-another-medicaid-cliff-for-puerto-rico/>
- [21] ASES, Plan Vital APR-DRG Frequently Asked Questions (March 2025). https://docs.pr.gov/files/ASES/APR-DRG/Plan%20Vital%20APR%20DRG_%20FAQ%20%20Final%2003-26-2025%20for%20ASES%20web.pdf
- [22] Center for Connected Health Policy, Puerto Rico telehealth and Medicaid policy summary. <https://www.cchpca.org/puerto-rico/>
- [23] Medicaid.gov, Rural Health Transformation Program eligibility. <https://www.medicaid.gov/resources-for-states/rural-health>
- [24] KFF, Medicaid postpartum coverage extension tracker (updated 15 July 2026). <https://www.kff.org/medicaid/medicaid-postpartum-coverage-extension-tracker/>
- [25] Centro de Periodismo Investigativo, First Medical unpaid claims and provider refusals (23 April 2026). <https://periodismoinvestigativo.com/2026/04/first-medical-unpaid-providers-puerto-rico-patients/>
- [26] Metro PR, "Quedó pillado incentivo contributivo del 4% para médicos" (16 May 2024). <https://www.metro.pr/noticias/2024/05/16/quedo-pillado-incentivo-contributivo-del-4-para-medicos/>
- [27] Microjuris, Ley 8-2025 telemedicine certification changes (20 April 2025). <https://aldia.microjuris.com/2025/04/20/profesionales-de-la-salud-en-puerto-rico-podran-ejercer-telemedicina-sin-certificacion-adicional/>
- [28] Ley 94-2023, liability protections for residents and trainees (LexJuris). <https://www.lexjuris.com/lexlex/leyes2023/lexl2023094.htm>
- [29] 9 Millones, "Tres leyes que restringen la salud reproductiva en Puerto Rico" (7 April 2026). <https://www.9millones.com/en/story/tres-leyes-que-restringen-la-salud-reproductiva-en-puerto-rico>
- [30] El Nuevo Día, organizations denounce new reproductive-health laws (20 April 2026). <https://www.elnuevodia.com/noticias/locales/notas/denuncian-que-nuevas-leyes-sobre-salud-reproductiva-agravan-crisis-de-servicios-medicos-en-puerto-rico/>
- [31] El Nuevo Día, senators demand action on unnecessary cesareans (17 September 2025). <https://www.elnuevodia.com/noticias/legislatura/notas/senadoras-del-ppd-y-pip-exigen-accion-para-enfrentar-alza-en-cesareas-innecesarias-en-puerto-rico/>
- [32] Blooming Families Clinic, summary of proposed birth measures including PC 669 (September 2025). <https://www.bloomingfamiliesclinic.com/post/nuevas-medidas-propuestas-sobre-parto-en-puerto-rico>
- [33] ProPublica, ERASE MM maternal mortality funding cuts (8 August 2025). <https://www.propublica.org/article/maternal-mortality-erase-mm-funding-cuts>
- [34] Commonwealth Fund, "What is PRAMS and why is it at risk" (January 2026). <https://www.commonwealthfund.org/publications/explainer/2026/jan/what-is-prams-and-why-is-it-at-risk>
- [35] Metro PR, Salud activates monitoring for congenital syphilis (26 February 2026). <https://www.metro.pr/noticias/2026/02/26/salud-activa-monitoreo-por-alza-de-sifilis-en-recien-nacidos/>
- [36] Sandall et al., Midwife continuity of care models versus other models of care, Cochrane Database (2024). <https://www.cochranelibrary.com/cdsr/doi/10.1002/14651858.CD004667.pub6/full>

- [37] Wernham et al., midwife-led versus medical-led care in New Zealand, PLOS Medicine (2016). <https://journals.plos.org/plosmedicine/article?id=10.1371%2Fjournal.pmed.1002134>
- [38] RIVM, limited effects after five years of bundled payments in Dutch maternity care. <https://www.rivm.nl/en/news/limited-effects-visible-after-5-years-of-bundled-payments-in-maternity-care>
- [39] MACPAC, Access to Maternity Providers: Midwives and Birth Centers (May 2023). <https://www.macpac.gov/wp-content/uploads/2023/05/Access-to-Maternity-Providers-Midwives-and-Birth-Centers.pdf>
- [40] NASHP, Medicaid financing of midwifery services: a 50-state analysis. <https://nashp.org/medicaid-financing-of-midwifery-services-a-50-state-analysis/>
- [41] NPEU, Birthplace in England study results. <https://www.npeu.ox.ac.uk/birthplace/results>
- [42] CMS, Strong Start for Mothers and Newborns final evaluation report. <https://downloads.cms.gov/files/cmml/strongstart-prenatal-finalevalrpt-v1.pdf>
- [43] National Academies, Birth Settings in America (2020). <https://www.ncbi.nlm.nih.gov/books/NBK555483/>
- [44] Hutton et al., perinatal or neonatal mortality among intended home births, eClinicalMedicine (2019). <https://www.sciencedirect.com/science/article/pii/S2589537019301191>
- [45] Bohren et al., Continuous support for women during childbirth, Cochrane (2017). https://www.cochrane.org/evidence/CD003766_continuous-support-women-during-childbirth
- [46] MACPAC, Doula in Medicaid: case study findings (November 2023). <https://www.macpac.gov/wp-content/uploads/2023/11/Doula-in-Medicaid-Case-Study-Findings.pdf>
- [47] Axios, doula care coverage by Medicaid and insurers (21 April 2026). <https://www.axios.com/2026/04/21/doula-care-medicaid-insurance>
- [48] ACOG Clinical Consensus No. 8, Tailored Prenatal Care Delivery (April 2025). <https://www.acog.org/clinical/clinical-guidance/clinical-consensus/articles/2025/04/tailored-prenatal-care-delivery-for-pregnant-individuals>
- [49] OB Nest cost analysis, BMC Pregnancy and Childbirth (2021). <https://link.springer.com/article/10.1186/s12884-021-03557-3>
- [50] Heart Safe Motherhood (Penn Medicine), evidence summary. <https://heartsafemotherhood.org/evidence>
- [51] Meta-analysis of remote blood pressure monitoring RCTs in postpartum hypertension, J Perinat Med (2025). <https://pubmed.ncbi.nlm.nih.gov/40123081/>
- [52] Chen et al., Non-clinical interventions for reducing unnecessary caesarean section, Cochrane EPOC (2018). https://www.cochrane.org/CD005528/EPOC_non-clinical-interventions-reducing-unnecessary-caesarean-section
- [53] Stanford Medicine, California's historic drop in cesareans (CMQCC). https://med.stanford.edu/obgyn/in-the-news/magazine-2023/historic_drop_csectionsocalifornia.html
- [54] Parto Adequado implementation evaluation, Reproductive Health (2024). <https://link.springer.com/article/10.1186/s12978-024-01773-6>
- [55] MACPAC, Medicaid payment initiatives to improve maternal and birth outcomes (April 2019). <https://www.macpac.gov/wp-content/uploads/2019/04/Medicaid-Payment-Initiatives-to-Improve-Maternal-and-Birth-Outcomes.pdf>
- [56] Fischer, Royer and White, rural hospital obstetric unit closures, NBER Working Paper 30141. https://www.nber.org/system/files/working_papers/w30141/w30141.pdf
- [57] AJMC summary of Kozhimannil et al., JAMA (2018), loss of rural obstetric services. <https://www.ajmc.com/view/loss-of-obstetric-services-in-rural-counties-associated-with-childbirth-risks>
- [58] Scottish community maternity units cohort, BMC Pregnancy and Childbirth (2017). <https://link.springer.com/article/10.1186/s12884-017-1374-9>
- [59] Family physicians providing obstetric care, JABFM (2023). <https://www.jabfm.org/content/36/4/565>

- [60] AIM, Obstetric Emergency Readiness Resource Kit. <https://saferbirth.org/aim-obstetric-emergency-readiness-resource-kit/>
- [61] MACPAC, Arkansas Perinatal Episode of Care (September 2021). <https://www.macpac.gov/wp-content/uploads/2021/09/Arkansas-Perinatal-Episode-of-Care.pdf>
- [62] MACPAC, Tennessee Perinatal Episode of Care Model (September 2021). <https://www.macpac.gov/wp-content/uploads/2021/09/Tennessee-Perinatal-Episode-of-Care-Model.pdf>
- [63] CMS Innovation Center, Transforming Maternal Health (TMaH) Model. <https://www.cms.gov/priorities/innovation/innovation-models/tmah>
- [64] NORC, Evaluation of the Pennsylvania Rural Health Model. <https://www.norc.org/research/projects/evaluation-of-the-pennsylvania-rural-health-model.html>
- [65] Center for Global Development, Millions Saved: Sri Lanka maternal mortality case. https://www.cgdev.org/sites/default/files/archive/doc/millions/MS_case_6.pdf
- [66] Guttmacher Institute, pharmacist-prescribed contraceptives state policy (April 2026). <https://www.guttmacher.org/state-policy/explore/pharmacist-prescribed-contraceptives>
- [67] UCSF, telehealth medication abortion safety study, Nature Medicine (February 2024). <https://www.ucsf.edu/news/2024/02/427111/telehealth-safe-visit-clinic-abortion-pills>
- [68] ASCO Post, FDA approval of at-home cervical screening self-collection (May 2025). <https://ascopost.com/news/may-2025/fda-approves-at-home-self-collection-device-for-cervical-cancer-screening/>
- [69] FDA Roundup, HPV self-collection in health care settings (17 May 2024). <https://www.fda.gov/news-events/press-announcements/fda-roundup-may-17-2024>
- [70] Butterfly Network, FDA clearance of Gestational Age Tool (2026). <https://www.butterflynetwork.com/press-releases/fda-cleared-gestational-age-tool>
- [71] AI-enabled gestational age estimation by novice users, JAMA Network Open (PMC). <https://pmc.ncbi.nlm.nih.gov/articles/PMC11350478/>
- [72] Handheld ultrasound price guide 2025 (Suresult). <https://suresultmed.com/handheld-ultrasound-price-guide-2025/>
- [73] Prevounce, 2026 remote patient monitoring CPT codes. <https://blog.prevounce.com/2026-remote-patient-monitoring-cpt-codes-whats-new-and-what-to-know>
- [74] Pomelo Care Medicaid virtual maternity cohort, Pregnancy (2026). <https://pubmed.ncbi.nlm.nih.gov/42596934/>
- [75] Mayo Clinic TeleOB program, Telemedicine and e-Health (2024). <https://pubmed.ncbi.nlm.nih.gov/38350119/>
- [76] Scoping review of provider-to-provider obstetric telehealth, Journal of Rural Health (2026). <https://pubmed.ncbi.nlm.nih.gov/41636340/>
- [77] E-MOTIVE cost-effectiveness, Nature Medicine (2024). <https://pubmed.ncbi.nlm.nih.gov/38844798/>
- [78] FDA licenses first freeze-dried plasma product (29 July 2026). <https://www.fda.gov/news-events/press-announcements/fda-licenses-first-ever-freeze-dried-plasma-product-us>
- [79] Zipline Rwanda postpartum hemorrhage mortality pre/post study (preprint). <https://www.researchsquare.com/article/rs-8454583/v1>
- [80] Drone Intelligence, FAA Part 108 status (September 2026). <https://droneintelligence.ai/insights/part-108-oira-operators-build-ahead>
- [81] Direct Relief, resilient power in Puerto Rico community clinics (January 2026). <https://www.directrelief.org/2026/01/in-puerto-ricos-community-clinics-resilient-power-bolsters-healthcare-and-access-to-treatment/>
- [82] Health Gorilla, Puerto Rico HIE commits to QHIN. <https://www.healthgorilla.com/blog/puerto-rico-hie-commits-to-joining-prospective-health-gorilla-qhin>

- [83] CompHealth, Interstate Medical Licensure Compact member list. <https://comphealth.com/resources/interstate-medical-licensure-compact>
- [84] NCSBN, Nurse Licensure Compact. <https://www.nursecompact.com/>
- [85] Barton Associates, OB/GYN salary and locum tenens guide. <https://www.bartonassociates.com/ob-gyn-physician-salary-guide/>
- [86] Lacted, integrating midwives into Puerto Rico's hospitals (August 2026). <https://www.lacted.com/post/integrar-parteras-al-sistema-de-salud-y-hospitales-de-puerto-rico-tiene-un-potencial-significativo-p>
- [87] Meta-analysis of continuous glucose monitoring in gestational diabetes, Acta Diabetologica (2026). <https://pubmed.ncbi.nlm.nih.gov/41591479/>
- [88] INFANT trial, computerized interpretation of fetal heart rate, Lancet (2017). <https://pubmed.ncbi.nlm.nih.gov/29437032/>
- [89] Bloomberg, Nuvo Group bankruptcy filing (22 August 2024). <https://www.bloomberg.com/news/articles/2024-08-22/newly-public-pregnancy-care-firm-nuvo-files-for-bankruptcy>
- [90] CDC, Zika Contraception Access Network (Z-CAN) in Puerto Rico. https://stacks.cdc.gov/view/cdc/110260/cdc_110260_DS1.pdf
- [91] Ley 122-2025 (LexJuris). <https://www.lexjuris.com/lexlex/Leyes2025/lexl2025122.htm>
- [92] WSIPP benefit-cost analysis, Nurse-Family Partnership. <https://www.wsipp.wa.gov/BenefitCost/Program/35>
- [93] CenteringPregnancy South Carolina Medicaid analysis (PubMed). <https://pubmed.ncbi.nlm.nih.gov/26979611/>
- [94] UAMS, ANGELS program evaluation. <https://idhi.uams.edu/high-risk-pregnancy-program/annual-reports/angels-program-evaluation/>
- [95] AHA, HRSA RMOMS final report highlights (24 July 2026). <https://www.aha.org/news/headline/2026-07-24-report-highlights-how-rural-areas-expanded-maternity-care-access>
- [96] Centro de Periodismo Investigativo, assisted reproduction without coverage (July 2024). <https://periodismoinvestigativo.com/2024/07/reproduccion-asistida-sin-cobertura-medica-maternidad/>
- [97] Systematic review of sFlt-1/PIGF cost studies, AJOG Global Reports (2025). <https://pubmed.ncbi.nlm.nih.gov/40496903/>
- [98] Act 60 Chapter 2 (formerly Act 14-2017) physician incentive summary (Torres CPA). <https://torrescpa.com/wp-content/uploads/2022/09/02-Professional-Physicians-Act-60-Chapter-2-of-Subtitle-B-formerly-known-as-Act-14.pdf>
- [99] Ley 186-2016, maternal mortality surveillance (OGP Biblioteca Virtual). <https://bvirtualogp.pr.gov/ogp/Bvirtual/leyesreferencia/PDF/186-2016.pdf>
- [100] Georgetown CCF, Consolidated Appropriations Act 2023 Medicaid provisions (January 2023). <https://ccf.georgetown.edu/2023/01/05/consolidated-appropriations-act-2023-medicare-and-chip-provisions-explained/>
- [101] University of Minnesota Rural Health Research Center, obstetric volume and quality in rural hospitals. <https://www.ruralhealthresearch.org/publications/1015>
- [102] El Vocero, "En jaque la obstetricia en Puerto Rico" (2023). https://www.elvocero.com/actualidad/salud/en-jaque-la-obstetricia-en-puerto-rico/article_04969f88-ff08-11ed-b497-174c34c20324.html
- [103] Wieggers TA, "Adjusting to motherhood: maternity care assistance during the postpartum period," Journal of Neonatal Nursing 12(5):163–171 (2006). Bibliographic reference supplied from general knowledge; not opened during verification.
- [104] Zorginstituut Nederland, kraamzorg entitlement under the basic health insurance package (statutory minimum of 24 hours over the first eight to ten days after birth). Reference supplied from general knowledge; not opened during verification. <https://www.zorginstituutnederland.nl/>

- [105] CMS, Puerto Rico State Plan Amendment PR-26-0001, APR-DRG inpatient payment methodology, effective 1 January 2026. <https://www.medicaid.gov/medicaid/spa/downloads/PR-26-0001.pdf>
- [106] ASES, Plan Vital APR-DRG Frequently Asked Questions, revised 3 February 2026. https://docs.pr.gov/files/ASES/APR-DRG/PR%20Plan%20Vital%20APR%20DRG_%20FAQ%20rev%2002-03-2026.pdf
- [107] ASES, APR DRG Base Rates by Hospital, effective 1 January 2026, version 3 (11 December 2025). <https://docs.pr.gov/files/ASES/APR-DRG/APR%20DRG%20Base%20Rates%20Effective%20Jan-01-2026.pdf>
- [108] ASES, Plan Vital APR DRG Reimbursement Guide, revised 15 January 2026. <https://docs.pr.gov/files/ASES/APR-DRG/ASES%20PR%20APR%20DRG%20Reimbursement%20Guide%20Rev%2001-15-2026.pdf>
- [109] Congressional Research Service, R47601, Medicaid financing for Puerto Rico and the other territories. <https://www.congress.gov/crs-product/R47601>
- [110] National Health Law Program (Doetzer), State directed payments and provider taxes after the 2025 reconciliation law (17 September 2025). https://healthlaw.org/wp-content/uploads/2025/09/Doetzer_NHELP_SDPs-and-PTs-after-OBBBA_091725.pdf
- [111] Puerto Rico Report, "How will the One Big Beautiful Bill Act impact Puerto Rico?" (2025). <https://puertoricoreport.com/how-will-the-one-big-beautiful-bill-act-impact-puerto-rico/>
- [112] CMS, Rural Health Transformation Program, eligibility (fifty states only; territories and DC not eligible). <https://cms.gov/priorities/rural-health-transformation-rht-program/rural-health-transformation-rht-program>
- [113] H.R. 6494, Territories Health Equity Act of 2025, 119th Congress, introduced 5 December 2025. <https://www.congress.gov/bill/119th-congress/house-bill/6494>
- [114] Metro PR, Governor and Resident Commissioner on Medicaid extension: \$4.4 billion minimum, 5% growth, 83% match (30 July 2026). <https://www.metro.pr/noticias/2026/07/30/gobernadora-y-comisionado-residente-confian-en-apoyo-bipartidista-para-extender-fondos-de-medicaid/>
- [115] Georgetown Center for Children and Families, postpartum coverage extension and Medicaid postpartum visit rates by program, FFY2020 (15 July 2022). <https://ccf.georgetown.edu/2022/07/15/opportunities-to-support-maternal-and-child-health-through-medicoids-new-postpartum-coverage-extension/>
- [116] Ley 156-2006, Ley de Acompañamiento durante el Trabajo de Parto, Nacimiento y Post-parto, as amended by Ley 200-2016 (LexJuris). <https://www.lexjuris.com/lexlex/leyes2006/lexl2006156.htm>
- [117] Ley 8-2025, amending Ley 168-2018 on telemedicine and telehealth: certification and payment parity for electronic, digital, and telephone services (LexJuris). <https://www.lexjuris.com/lexlex/Leyes2025/lexl2025008.htm>
- [118] Centro de Periodismo Investigativo, "Cuando la geografía determina tu acceso a servicios maternos" (May 2026), Spanish edition of [1]. <https://periodismoinvestigativo.com/2026/05/geografia-determina-acceso-servicios-embarazadas/>
- [119] Metro PR, deliveries become the leading reason for admission at the Hospital Universitario de Adultos (13 September 2026). <https://www.metro.pr/noticias/2026/09/13/partos-se-convierten-en-la-razon-principal-de-admision-en-el-hospital-universitario-de-adultos/>
- [120] Asociación de Hospitales de Puerto Rico, Departamento de Salud evaluates delivery-ward closures; 29 hospitals with maternity wards (January 2024). <https://www.hospitalespr.org/departamento-de-salud-evalua-situacion-de-las-salas-de-parto-ante-continuos-cierres/>
- [121] NotiCel, Vieques hospital inauguration still planned for end of September 2026; operator negotiation; FAA-certified helipad (28 August 2026). <https://noticel.com/>
- [122] FEMA, reconstruction begins at the Vieques health facility: \$85.6 million project, \$39.57 million FEMA share (15 November 2023). <https://www.fema.gov/es/press-release/20231115/reconstruction-begins-vieques-health-facility>

- [123] Caribbean Medical Center, Fajardo, labor and delivery service description.
<https://caribbeanmedicalcenter.com/sala-de-partos/>
- [124] NCHS, Births: Final Data for 2024, National Vital Statistics Reports 75(2) (9 June 2026).
<https://www.cdc.gov/nchs/data/nvsr/nvsr75/nvsr75-02.pdf>
- [125] March of Dimes PeriStats, Puerto Rico prenatal care, preterm, and low birthweight indicators (2023 data).
<https://www.marchofdimes.org/peristats/data?reg=99&top=5&stop=20&lev=1&slev=4&obj=3&sreg=72>
- [126] CEPAL, total fertility rate in Latin America and the Caribbean reaches 1.8 children per woman in 2024; Puerto Rico 0.94, lowest in the region (2025). <https://www.cepal.org/en/pressreleases/total-fertility-rate-latin-america-and-caribbean-reaches-18-children-woman-2024>
- [127] Todas, "Cuando la cesárea deja de ser elección: el caso de Puerto Rico" (Departamento de Salud 2023 cesarean rate 50.6%). <https://todaspr.com/cuando-cesarea-deja-eleccion-caso-puerto-rico/>
- [128] Todas, late prenatal care and preventable birth complications: severe maternal morbidity 307 per 10,000 deliveries versus 93.1 nationally (2026). <https://todaspr.com/atencion-prenatal-tardia-complicaciones-parto-prevenibles/>
- [129] El Vocero, abortion counts reported by the Departamento de Salud: 4,201 (2018), 3,596 (2019), 3,751 (2020) (November 2021). <https://www.elvocero.com/>
- [130] Centro de Periodismo Investigativo, anti-abortion centers and women's health in Puerto Rico; four regulated clinics (September 2024). <https://periodismoinvestigativo.com/2024/09/anti-abortion-centers-women-health-puerto-rico/>
- [131] Metro PR, Governor signs Ley 122-2025 (PS 297) on parental consent for abortion at age 15 and under (30 October 2025). <https://www.metro.pr/noticias/2025/10/30/>
- [132] EWTN News, Puerto Rico's Penal Code recognizes the unborn as human beings: Ley 18-2026 (PS 923), signed 12 February 2026. <https://www.ewtnnews.com/world/us/puerto-ricos-penal-code-recognizes-unborn-babies-as-human-beings>
- [133] Center for Reproductive Rights, abortion laws by state: Puerto Rico.
<https://reproductiverights.org/maps/abortion-laws-by-state/puerto-rico/>
- [134] Departamento de Salud de Puerto Rico, Orden Administrativa 585, mandatory syphilis screening in each trimester and at delivery (18 January 2024). <https://www.salud.pr.gov/CMS/DOWNLOAD/8485>
- [135] AMA FREIDA, San Juan City Hospital Obstetrics and Gynecology Residency Program (ACGME 2204212267), four positions per year. <https://freida.ama-assn.org/program/2204212267>
- [136] AMA FREIDA, Hospital Episcopal San Lucas / Ponce Health Sciences University Obstetrics and Gynecology Residency Program (ACGME 2204221346), five positions per year.
<https://freida.ama-assn.org/program/2204221346>
- [137] El Nuevo Día, "Claman por insertar la partería en el sistema de salud": 196 women in home-based midwifery care and 171 home births in 2021. <https://www.elnuevodia.com/noticias/locales/notas/la-parteria-como-alternativa-para-asegurar-los-derechos-de-las-personas-gestantes/>
- [138] Junta Examinadora de Enfermeras y Enfermeros de Puerto Rico, form JEE-005, advanced practice license requirements (revised February 2024). <https://www.salud.pr.gov/CMS/DOWNLOAD/8078>
- [139] SIMED, primary professional liability policy limits (\$100,000 per incident, \$300,000 aggregate for physicians).
<https://simedpr.com/polizas-primaria/>
- [140] Ley 229-2004, extending Ley 104-1955 protection to obstetrics, orthopedics, general surgery, and trauma physicians in public institutions (LexJuris). <https://www.lexjuris.com/lexlex/Leyes2004/lexl2004229.htm>
- [141] Ley 102-2025, Ley de Reconocimiento Universal de Licencias Ocupacionales y Profesionales (1 August 2025) (LexJuris); proposed Departamento de Salud regulation, December 2025.
<https://www.lexjuris.com/lexlex/Leyes2025/lexl2025102.htm>

- [142] Metro PR, DDEC recognizes the physician tax incentive as harmful to Puerto Rico; average beneficiary income \$270,000 (22 May 2024). <https://www.metro.pr/noticias/2024/05/22/ddec-reconoce-incentivo-para-medicos-como-pernicioso-para-puerto-rico/>
- [143] 13 L.P.R.A. § 45152 (Ley 60-2019), physician decrees after 31 December 2020 conditional on a Departamento de Salud certification of pressing need (Justia). <https://law.justia.com/codes/puerto-rico/2020/titulo-13/subtitulo-19/parte-ii/capitulo-1304/subcapitulo-c/45152/>
- [144] Oficina de Presupuesto de la Asamblea Legislativa, Informe 2026-271 on PS 849 and PC 332 (fiscal impact of reactivating physician decrees). <https://www.opal.pr.gov/informes/informe-2026-271-ps-849-y-pc-332>
- [145] Metro PR, Departamento de Salud endorses PS 15 on physician retention and recruitment with fiscal caveats (24 March 2026). <https://www.metro.pr/estilo-vida/2026/03/24/salud-favorece-medida-legislativa-para-fortalecer-la-retencion-y-el-reclutamiento-de-medicos-en-la-isla/>
- [146] USCIS, Conrad 30 Waiver Program (30 recommendations per year per state, DC, Guam, Puerto Rico, and the US Virgin Islands). <https://www.uscis.gov/working-in-the-united-states/students-and-exchange-visitors/conrad-30-waiver-program>
- [147] Council of State Governments, Interstate Medical Licensure Compact membership (44 states, DC, and Guam after Alaska's accession, June 2026). <https://compacts.csg.org/compact/interstate-medical-licensure-compact/>
- [148] Compact States, Nurse Licensure Compact status by jurisdiction (43 enacted, 40 implemented, July 2026). <https://www.compactstates.com/compact-nursing-states>
- [149] Patria Nueva, the unnecessary cesarean rate in Puerto Rico and the legislative history of physician-level reporting bills since 2008. <https://www.patrianuevapr.com/post/la-tasa-de-ces%C3%A1reas-innecesarias-en-puerto-rico-ha-continuado-en-aumento>
- [150] Periódico La Perla, PIP senators file measures to reform maternal health (PS 445 and related bills, March 2025). <https://www.periodicolaperla.com/actualidad/senadores-del-pip-presentan-medidas-para-reformar-la-salud-materna/>
- [151] Plural Policy, PS 435 (2025), right to direct OBGYN care through twelve months postpartum. <https://open.pluralpolicy.com/pr/bills/2025-2028/PS435/>
- [152] NotiCel, Professional Hospital management denounces rate failures and warns ASES (7 April 2026). <https://noticel.com/noticias/20260407/gerencia-de-professional-hospital-denuncia-fallas-en-tarifas-y-lanza-advertencia-a-ases/>
- [153] Metro PR, hospital sues ASES in federal court over reimbursement disparities (13 May 2026). <https://www.metro.pr/noticias/2026/05/13/hospital-demanda-en-foro-federal-a-ases-por-disparidad-en-reembolsos>
- [154] FDA 510(k) summary K252148, Butterfly Network Gestational Age Tool, decision 27 March 2026 (unverified pointer; summary not opened). https://www.accessdata.fda.gov/cdrh_docs/pdf25/K252148.pdf
- [155] Microjuris, new law authorizes provisional licenses for health professionals during emergencies (15 July 2026). <https://aldia.microjuris.com/2026/07/15/nueva-ley-autoriza-licencias-provisionales-para-profesionales-de-la-salud-durante-emergencias/>